UNITED STATES DISTRICT COURT

SOUTHERN DISTRICT OF CALIFORNIA

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| UNITED STATES OF AMERICA, | ) | |
| | ) | |
| Plaintiff, | ) | Case No. 21-cr-1289-TWR |
| | ) | |
| VS. | ) | San Diego, California |
| | ) | |
| NAMEER MOHAMMAD ATTA, | ) | Monday, |
| | ) | September 26, 2022 |
| Defendant. | ) | 8:34 a.m. |
| _____ | ) | |

REPORTER'S TRANSCRIPT OF PROCEEDINGS

JURY TRIAL - DAY ONE (Pages 1 - 195)

BEFORE THE HONORABLE TODD W. ROBINSON
UNITED STATES DISTRICT JUDGE

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San Diego, California - Monday, September 26, 2022

8:34 a.m.

THE CLERK: Calling case 21-cr-1289, U.S.A. vs. Nameer Atta for jury trial.

Counsel.

MS. FORREST: Good morning, Your Honor. J'me Forrest and Owen Roth -- who stepped out for a moment but will be right back -- for the United States.

THE COURT: Okay. Good morning.

MR. CORTEZ: Good morning. Ezekiel Cortez on behalf of Mr. Atta, who is present in custody.

At counsel table, we have Ralph Cordova. He's not going to do anything. He's going to be an inanimate object. Keeping me in line.

THE COURT: Are you going to introduce him to the jurors?

MR. CORTEZ: Yes. In case they know him.

THE COURT: Okay.

MS. FORREST: Your Honor, we have some non-inanimate objects at our table, people who will be doing some things. We have Lina Douglas, as well as the case agent, Stephen Logan.

THE COURT: Okay. Good morning to you, both.

Okay. First things first, Mr. Cortez.

MR. CORTEZ: Yes.

THE COURT: It's my intention to discuss some of the in-limine rulings and some of the pending motions.

MR. CORTEZ: Yes, please.

THE COURT: Do you waive the presence of Mr. Atta to to discuss those legal matters?

MR. CORTEZ: I do.

THE COURT: Okay. And for the record, Mr. Roth has joined us now.

Okay. Going through the in-limine motions, I read the transcript, I've read the parties' motions. I've also gone back and read some of the other transcripts, so I think I'm fully up to speed on the issues that are still pending.

And in that regard, I'm going to go through the in-limine motions and just tell you what my understanding is and clarify anything that was left open for my consideration by Judge Sammartino.

The first one is to the -- admit the expert testimony. And there was a grant with respect to Dr. Platt, Special Agent, is it Klinge?

MR. ROTH: Klinge, yes, your Honor.

THE COURT: Okay. And the analysts, Lim and Evans. And there is a reserved ruling on the motion with respect to Criminalist Christopher Vance. And I would also reserve on that pending the evidence coming in.

The evidence that Mr. Vance would testify to, as I

understand it, are sales to other individuals involving pills dissimilar to the ones allegedly sold to the decedent in this case.

Is that a correct understanding?

MR. ROTH: In part, Your Honor. However, if helpful, we're not going to call him; so we can moot this particular expert.

THE COURT: Okay.

MR. ROTH: I'm sorry -- I apologize to the Court for not updating you as to the particulars over the weekend.

THE COURT: Okay.

MR. ROTH: But we're not going to call Christopher Vance, and we're not going to introduce evidence of those sales of those pills.

MR. CORTEZ: And counsel updated me over the weekend.

THE COURT: Terrific. I appreciate the parties working with one another.

MR. CORTEZ: Your Honor, for the record, we just received the exhibits with numbers. We've had them over the weekend, but not with numbers; so that's why I'm going through the numbers right now.

THE COURT: Okay. In-limine No. 2 was a grant, and I would adopt that.

The third was admit the text messages between

Mr. Atta and third parties highlighting his understanding of fentanyl-laced pills. I agree with Judge Sammartino's tentative on that. The conversations actually have to go to the defendant's knowledge of fentanyl-laced pills. So with that in mind, I will rule on any objections as they come.

The geolocation data was No. 4. I would adopt Judge Sammartino's ruling on that. That was a grant.

The social media, No. 5, I would adopt Judge Sammartino's ruling on that. That was a grant in part. I agree with Judge Sammartino that general evidence about Mr. Atta being a prolific drug dealer is something that is properly excluded, but I fully intend to allow the government to present evidence with respect to fentanyl-laced pills and other individuals.

So that would be -- yes.

MR. ROTH: Let me -- if I may, Your Honor. I can give the Court some more precise explanation of what the evidence will be as to that, that way at least the Court is prepared for when objections come, if they do.

There are, I think the best way to describe it, sort of like rings on a dart board, concentric circles of evidence here. Tapped at center, evidence of sales of the pills to the defendant that led directly to his death, sales of pills --

THE COURT: To the decedent.

MR. ROTH: The decedent. I'm sorry. I'm sorry.

Just outside it, sales of pills to the victim over the course of about a five-week period that culminated in his death. Outside of that, sales of pills to other people. And then outside of that, sales of other drugs to other people.

THE COURT: Okay. You've lost me on the last ring.

MR. ROTH: Sales of other drugs to other people. Sales of mushrooms and marijuana to people who are not the victim.

As to the most outermost ring, Judge, we really have made our very best effort to get rid of all of it. There are a few items of evidence in the form of Snapchat posts of menus that refer to a variety of drugs. We intend to offer a variety of them. Some list Percocet; some don't. But that's really the extent of the evidence that could reveal, in any direct fashion, sales of other drugs to other people.

With respect to sales of Percocet in general, we do have evidence. We have text messages discussing Percocet sales. We have video posted to Snapchat that doesn't refer to any drug in particular, but talks about the trap being open for business, things like that.

And then with respect to sales of pills to the victim, both in the general sense and precisely, we're putting in everything we have that's admissible. So we've

tried to really pare this down. It's not perfect because we kind of can't do it, as Your Honor will see. But that's the gist of the case.

THE COURT: Okay. With that clarification, Mr. Cortez, do you still lodge an objection to the messages or the postings that reference other drugs?

MR. CORTEZ: I do.

THE COURT: Okay. I'm going to have to take a look at those. Can those be redacted?

MR. ROTH: If Your Honor wants to see them, they are Exhibit 42 of the Court's binder. It's A through G.

THE COURT: Okay.

MR. ROTH: And yes, we can redact them. I would ask for the Court's guidance, and I would just ask for a little bit of latitude on redacting ones that have Percocet in them. Perhaps a more precise way to do it would be to only introduce the posts that reference Percocet.

MR. CORTEZ: Mr. Atta is now before the Court in custody.

THE COURT: Okay.

MR. ROTH: It's also worthy of note, Judge, on one instance the defendant bought a drug that was not Percocet. It's a single instance. It's otherwise relevant, and it is in the evidence -- I'm sorry. The defendant sold to the decedent.

THE COURT: Okay.

MR. CORTEZ: I'm sorry.

THE COURT: Mr. Cortez, with respect to the menu items, that is, Government's Exhibit 42A, B, C, D, E, F and G, I leave it up to you whether or not your request is that they redact it. The jurors could, if they are redacted, draw an inference from the redacted versions. I can give a limiting instruction that they are only to consider the evidence with respect to the Percocet and any fentanyl, and any other drug dealing is not relevant for their consideration. So I leave it up to you as a tactical matter.

I believe the government is entitled to show that Mr. Atta had for sale the Percocet and those items that pertain directly and are tethered to this particular case. But do you have a request with regard to redacting those exhibits that I just referenced?

MR. CORTEZ: I will reemphasize my objection to all of that material and why. Because the charge in this case, as the government even noted in its papers, doesn't require them to prove intent or mental condition -- mental intent to sell fentanyl or Percocet or any specific controlled substances.

Having noted my objection, again, I would, of course, given the Court's ruling, I would ask for a redaction.

THE COURT: So going to the intent, the government must prove that Mr. Atta had the intent to sell Percocet, fentanyl or some other federally controlled substance.

Would not the unredacted versions of these go to that intent that the government must prove?

MR. CORTEZ: No, Your Honor. Our position, which is is in our moving papers, based on the government's own representation to the Court, to Judge Sammartino, and quoting of the Ninth Circuit Jury Instruction, they don't have to prove intent of the drug.

THE COURT: They don't have to prove the intent as it relates to the death resulting, but they do have to prove intent with regard to the distribution of a controlled substance.

MR. CORTEZ: The way we read it -- and it's very explicit in the jury instructions -- they don't have to prove foreseeability that the controlled substance will cause death. And -- actually, they do. That's exactly what they have to prove, that without foreseeability, that Mr. Atta sold a controlled substance --

THE COURT: But knowingly and intentionally sold a controlled substance --

MR. CORTEZ: Oh, absolutely. Absolutely. To Mr. Reed. To Mr. Reed.

THE COURT: So that's the intent that I'm talking

about at this point --

MR. CORTEZ: Right.

THE COURT: -- the intent to distribute a controlled substance, and then the resulting in death is a separate intent.

So focusing only on the intent to distribute a controlled substance, wouldn't all of these controlled substances go to his intent?

MR. CORTEZ: I would argue no. As I noted in my objections. Having heard the Court's rulings, Sammartino's and your tentative, I would object; but I would invite a redaction of other drugs.

THE COURT: Okay. What is the government's position?

MR. ROTH: Your Honor, I would propose that in lieu of redactions, that the government forego introducing Government's Exhibits D, E and F, which are menus that do not reference Percocet. We can make the point we wish to make without those three, and I think that has the effect of, frankly, narrowing 42 by basically half.

I share the Court's concern that redactions are worse than not redactions, because it would invite the jury to wonder what's under those redactions. And Mr. Atta did not sell heroin. He did not sell crack. He did not sell a number of drugs that are known to be exceedingly dangerous

and addictive. And if a jury is wondering, I think that could be prejudicial.

So I think if we remove those three, and we limit it to -- it's really an exhibit of three examples of menus, because one of them we're showing an update to, if Your Honor compares A and B. It's three menus from the defendant's phone listing products, including Percocet. We can make the point that he understood Percocet was a drug that existed in the market that he could sell for profit, and that's really our point.

THE COURT: Okay. What is the difference between 42C and 42A and B? I understand that B is an updated version of A. What is the relationship between those two exhibits and C?

MR. ROTH: Your Honor, it's another version of a menu. If Your Honor looks at the bottom half where it says "non-THC product list," Your Honor will see "percs" are listed.

Our point is we intend to argue the defendant marketed, advertised, promoted sales of Percocet via Snapchat. We want to show a few different examples of him doing that to make the point, this was his business practice. We think that goes directly to knowing distribution, as Your Honor has pointed out is an element, and knowledge that it's drugs, controlled substances.

THE COURT: Okay.

MR. ROTH: And not sugar pills.

THE COURT: Mr. Cortez, the government is self-limiting to Exhibits 42A, B and C at this point.

MR. ROTH: I'm sorry. Also G.

MR. CORTEZ: G.

MR. ROTH: Which also, as Your Honor will see, lists Percocet.

Your Honor, there is an independent value to this one. If Your Honor looks at the symbol, or emoji for Perc 30s in 42G, Your Honor will see it's a blue circle with a white "M" in it. In the early menus, it's a blue diamond. We have evidence in the form of a ledger that the defendant kept referring to Percocet sales to the victim and others, and he categorizes them by emoji. And those are the interchangeable emojis he uses. So this goes to corroboration of sales.

THE COURT: Okay.

MR. CORTEZ: I don't want to go back and forth. Just note my objections. That's all I have to say.

THE COURT: Okay. So with the government's withdrawal of the other exhibits, my tentative with respect to 42A, B, C and G is to admit those exhibits and give a limiting instruction to the jury reminding them of what the charge is in this case and for what purpose they can consider

those exhibits. Okay.

So next up is No. 7, which was the video footage, the body-worn camera video footage. I think at our last hearing, I noted that I had watched the video footage. Judge Sammartino did not have the benefit of watching that video footage. I find that the video footage is relevant under Rule 401. However, I also need to analyze that evidence under Rule 403.

And in that regard, I am cognizant of the government's argument that the comments that are being made during the time period that the EMTs and the paramedics were administering treatment to the decedent are relevant. And the government is not limited in its proof just to the testimony of the ME.

However, I believe that the video footage is such that its relevance is substantially outweighed by the prejudice that would flow if the jury was exposed to that particular very graphic and very disturbing images and video footage of the decedent in the state that he was when treatment was being administered.

With that having been said, a lot of the weighing -- and it is a balancing test -- will depend upon the evidence that comes in at trial. So this is subject to revision.

Mr. Cortez, I don't know specifically what defense or defenses you will be pursuing in this case, but if you do

challenge the testimony or the government's evidence that the fentanyl was the cause of death, as opposed to other contributing factors, that will weigh into the 403 balancing test as to the probative nature of the evidence versus the prejudicial aspect of it.

So I'm trying to be as transparent as possible with respect to my weighing of those competing concerns, but at this point in time, the government's motion to introduce the video footage is respectfully denied. And, of course, that's subject to revisit -- to being revisited during the course of trial.

I will permit the government -- in addition to the video footage, there were still shots. There were two still shots. There was a still shot where the victim had the machine on his chest that was doing the compressions. That, I don't find that that is particularly relevant, but I will allow the government to introduce the still shot of the EMT or the paramedic administering the breaths. And I think that that is -- under 403, as I said, the video itself was relevant, but I think that takes care of my overriding 403 concerns with respect to the prejudicial nature.

The government is entitled to prove its case, and one of the things that they have to prove is that the victim in this case suffered a fatal overdose. And the condition of the victim when the paramedics and the EMT responded is

relevant. And I find that the one picture, and I've got a copy of that picture because multiple ones were submitted to me, and I'll have Ms. Ortiz hand this to the parties, so that we're all talking about exactly the same thing. I find that that is admissible under 403. So that's my ruling in that regard.

MR. CORTEZ: Thank you.

MR. ROTH: Your Honor, just to complete the record. There are two images, and Your Honor has identified Government's Exhibit 2.

THE COURT: Okay.

MR. ROTH: Thank you. Government's Exhibit 1 is similar. We would just ask for a ruling.

THE COURT: Okay. 1, I find that it's cumulative. The probative value of the photograph is the condition of the decedent when the paramedics and EMTs were administering treatment to him, so I find that Government Exhibit 1 is cumulative to Government Exhibit 2.

So while I will allow at this point Exhibit 2 in, 1 I find is not properly admitted.

MR. ROTH: Okay. Thank you, Your Honor.

THE COURT: Okay. Eight, testimony from the witness identified as A.G. pertaining to the sale of fentanyl pills. That was granted. I adopt that.

MR. ROTH: It's moot, Your Honor. A.G. will not

testify. That is the person who was the subject of the supplemental motion that we withdrew late last week.

THE COURT: Okay. No. 9 was tied in with the criminalist, so I believe that's denied as moot.

As to the excited utterances, government in-limine 10, I adopt Judge Sammartino's tentative on that.

The motion filed, the in-limine motion filed as No. 11, the aspects of the medical history, Judge Sammartino's ruling as I understand it was that the medical history of the decedent may properly be inquired into on cross-examination of the medical examiner as to the medical examiner's ultimate opinion that the death resulted from fentanyl, as opposed to other causes. And I adopt that tentative. I believe that that's a proper line of cross-examination.

No. 12, I would adopt the tentative on that with respect to the Wounded Warrior Project being granted. That motion regarding the Wounded Warrior Project being granted. And the A.G. is, of course, a moot point at this point, the testimony regarding that.

13 --

MR. CORTEZ: Your Honor --

THE COURT: Yes.

MR. CORTEZ: Although not entirely. The government wants to offer Agent Logan's report of her interview.

MR. ROTH: No, we don't.

THE COURT: Okay.

MR. CORTEZ: Okay. See. We're catching up.

THE COURT: We're narrowing the issues, and I appreciate that.

MR. CORTEZ: Yes.

THE COURT: With respect to Motion in Limine No. 13, the ruling by Judge Sammartino was reserved on that, and it depends on the evidence at trial. I agree. We'll see how the evidence comes in as to what other sources of fentanyl or any other controlled substance the decedent might have had.

14 was granted. That's the arrest of Mr. Atta for selling a gram of cocaine. I adopt that tentative.

Any references to punishment are impermissible, and I adopt that. That was No. 15.

And the motion filed as 16 was the unrelated information about Mr. Atta that was granted. And I adopt that, unless some relevance is established by the defense.

And 17, again, I adopt that. If there has been any discovery-related incidences or alleged violations, I'll address those as they go.

And Mr. Cortez, do you intend to call experts at this point --

MR. CORTEZ: No.

THE COURT: -- or do you reserve on that?

MR. CORTEZ: I'm going to reserve.

THE COURT: Okay. And the final is the government's new in-limine motion to preclude evidence that the decedent engaged in the use of acid in the fall of 2019.

I believe that ties in with No. 11, and my tentative is to grant that with respect to character evidence of the decedent. But Mr. Cortez, I believe that you can properly inquire of the ME whether use of acid in the fall of 2019 may or may not have been a contributing factor to the death of C.M.R., as opposed to the opinion, as I understand it, that it was solely attributable to the fentanyl that he had ingested.

MR. CORTEZ: Thank you.

THE COURT: Okay. Does that take care of all the in-limine motions and all pending motions? From the government's perspective?

MR. CORTEZ: It does.

MR. ROTH: Yes, Your Honor. Thank you.

THE COURT: Mr. Cortez?

MR. CORTEZ: For the record, I will just restate my objections.

THE COURT: Okay. Of course.

MR. CORTEZ: And with that, we're ready.

THE COURT: Okay. That brings me next to --

MR. ROTH: Your Honor, may I confer with counsel for

just a moment?

MR. CORTEZ: He conferred all weekend with me. I protest.

(Attorney conference.)

MR. ROTH: Your Honor, pursuant -- in this case, the discovery includes two phones that were seized from the defendant. They are largely duplicative. Pursuant to an agreement between the parties, the United States will not be introducing any evidence from one of those phones for any purpose, including cross-examination.

For the record, that is the phone that was seized from Mr. Atta in late April of 2021. About two weeks later, I think it's about May 5th, a second phone was seized from him. All government evidence concerning the defendant's phone as referenced in front of the jury will be a reference to the second phone.

THE COURT: Okay.

MR. ROTH: Thank you.

THE COURT: Okay. With regard to the preliminary jury instructions, Mr. Cortez and the government also filed their preliminary instructions.

Mr. Cortez, you made reference in your filing that you were adopting all of the ones that were filed by the government. In going through and comparing and contrasting, there were a couple of instances where the government

included something that you did not include.

MR. CORTEZ: Correct.

THE COURT: And I want to make sure that you're okay with those aspects of it.

MR. CORTEZ: Thank you for asking. The only ones I have an issue with is the ones that appear to get into closing argument and the law regarding circumstantial evidence and what the government's burden is.

THE COURT: No, no, no. This isn't the voir dire, the submitted voir dire questions. This is the preliminary instructions to the jury. These are the pattern.

MR. CORTEZ: Oh, jury instructions. We all agree.

THE COURT: Okay. You said that yours were the same as the government, and that was not entirely true. And I just wanted to highlight the two instructions where the government included language that you did not include and want to make sure that you're okay with it.

MR. CORTEZ: Yes.

THE COURT: Jury Instruction No. 2, the government amended the pattern instruction to include reference to C.M.R. and the statutes that are alleged to have been violated by Mr. Atta; that is, 841(a)(1) and then the penalty provision of (b)(1)(C). And then the government also included a very brief recitation of the elements that they must prove to put the evidence in context.

And my tentative is to grant the request to give those instructions. I believe that it's appropriate for the jurors to know what the government's burden of proof is as they listen to the evidence in this case.

MR. CORTEZ: I agree.

THE COURT: Okay. There was also Jury Instruction No. 11, and there is an italicized portion in the second paragraph where I'm going through and instructing the jury as to what happens during the course of trial, and when I reference then if the defendant chooses to offer evidence, counsel for the government may cross-examine, there is an italicized portion, the defendant has no duty at all to offer any evidence.

And Mr. Cortez, my question in that regard is if you wish me to give that instruction, given your plan to present or not present any evidence?

MR. CORTEZ: At this point, I am going to reserve commenting.

THE COURT: Okay. Do you request that I give that or not give that, or do you leave that to my discretion?

MR. CORTEZ: I'll just submit it to the Court.

THE COURT: Okay. The pattern instructions that the jury will receive at the end of the case make reference to the fact that at no time does the defendant have any obligation to present any evidence or testify, and I believe

that accurately covers that particular instruction, so my tentative is not to give the italicized portion.

MR. CORTEZ: Thank you, Your Honor.

THE COURT: Okay. So the voir dire questions, I know I gave the parties 15 minutes for voir dire. There were a number of questions that not only tracked the standard questions that are posed to the jurors, but there were some that were submitted that I think are fairly characterized as argumentative in nature and go to the ultimate deliberative process, if you will, of the jurors and somewhat asks them to pre-judge some of the evidence.

So out of the submitted questions, I've selected two -- or I've created two. And the first one goes to the nature of the charge in this case, and whether given the nature of the charge, any of the jurors simply could not be fair and impartial to both sides. So it is a recitation of the charge and it incorporates a comment on the distribution of controlled substance as well as the evidence that they will hear regarding that the use of a controlled substance is alleged to have resulted in the death of C.M.R.

So I do intend to inquire as to that so that the parties need not sort of associate themselves with those particular issues.

MR. CORTEZ: Thank you.

THE COURT: The second was the standard questions

that I ask of all the jurors that have to do with a relationship with law enforcement and whether they would give a law enforcement officer's testimony greater or less credibility just by virtue of their employment status.

The second question tailored to the specific facts of this case has to deal with military personnel testifying in this case and whether by virtue of their status as military personnel, whether any of the jurors would give that testimony greater or lesser credibility just by virtue of their status as a member of the military.

So those are the two questions that I culled from the parties' suggested questions. This is a unique case and raises issues that we don't often see. I will allow the parties 20 minutes of attorney-conducted voir dire. I think that from the questions that were submitted, there are a number that are properly put to the jurors, but are not so sensitive in nature that the Court needs to ask those particular questions.

So I defer to counsel on both sides as to which of the proposed questions they want to inquire during the attorney-conducted voir dire, but those other two areas that I previously mentioned, I think are properly addressed by the Court so that the parties don't have to identify themselves with those more sensitive issues.

MR. ROTH: Your Honor, if the Court's view of the

argumentative proposed voir dire is that the substance is not properly acquired by voir dire, would the Court mind flagging for the parties which ones, so that we don't do it. If it's a matter of tone, we will just do our voir dire.

THE COURT: It's not a matter of tone. It's a matter of, you know, will you vote guilty if the government proves its case beyond a reasonable doubt; and there was a similar submission by the defense.

So any questions going to the ultimate issue that the jurors must decide, or their consideration of any particular evidence in the case, I believe that those lines of inquiry would be improper during the attorney-conducted voir dire.

So without going through each and every one of those and giving you my tentative as to each and every one of those, keeping those parameter in mind. We have very experienced counsel on both sides of this case, and I'll take any objections as they are lodged.

MR. ROTH: Thank you, Judge.

MR. CORTEZ: Thank you.

THE COURT: Okay. With regard to any other matters that we need to address, is there anything else from the government's perspective before we bring the jurors in?

MR. ROTH: No, Your Honor.

THE COURT: Okay. From the defense?

MR. CORTEZ: No. I had an issue before with the marshals sitting between my client and one of us, but that's not going to happen. He looks pretty good in that suit. Thank you.

THE COURT: Okay. It's been my experience that the marshals who are in the court are very professional and sort of meld into the scene and are not independently identifiable as law enforcement personnel.

So when we bring the jurors in, just --

THE MARSHAL: I'm going to hop up to the table, sir.

THE COURT: That will be terrific. So we're going to put the jurors where the questionnaires are already in the seats. So I don't anticipate any issue.

We've got a number of people in the gallery. I would caution you not to have any interactions at any time with any of the jurors. This case will be tried in a fair and impartial manner, and I don't want anyone to have contact with any of the jurors; nor will there be any comment on any of the evidence as it comes in. And I hope that is clear to all the people in the gallery.

With that in mind, we're going to bring the jurors in, and we'll begin the jury selection process.

(Recess, 9:05 a.m. to 9:20 a.m.)

THE CLERK: Calling case 21-cr-1289, U.S.A. vs.

Nameer Mohammad Atta for jury trial.

Counsel.

MR. ROTH: Good morning, Your Honor. Owen Roth, joined by J'me Forrest and Lina Douglas on behalf of the United States.

THE COURT: Good morning.

MR. CORTEZ: Ezekiel Cortez for Nameer Atta. And Ralph Cordova, who is going to be assisting at counsel table.

THE COURT: Okay. Thank you and good morning.

MR. CORDOVA: Good morning.

THE COURT: Ladies and gentlemen, I want to start this morning by welcoming all of you to the courthouse. And I want to thank you sincerely for your willingness to participate in this incredibly important civic duty that we all have as citizens of this country.

Our criminal justice system could not do what it needs to do without the willingness of each and every one of you to take time out of what I appreciate and understand are very busy schedules to come here to the courthouse and participate as prospective jurors in the jury selection process here this morning. So I want to thank each and every one of you for your willingness to do that.

I also want to talk very briefly about the pandemic and our current posture with regard to the pandemic. We

follow all of the CDC and local health guidelines, and I am pleased to let each and every one of you know that throughout the pandemic, we have managed to stay open and conduct business as a court over the last two years. And we've done so by changing method of operating jury trials. We are back to our normal method of operation at this point.

But I am pleased to let you know that despite the fact that we've been open throughout the pandemic, I have received no information that any of the jurors who have participated in any of the trials have had negative consequences as a result of performing this very important duty as jurors.

So we do take your health and safety very seriously. We clean the courtroom twice a day. And we leave it up to each and every one of you, because that's the current guidance, as to whether or not you choose to wear a mask or choose not to wear a mask. You're free to do either one throughout the proceedings.

So I did want to start with that preliminary remark about your health and safety, and, again, thank you very much for your willingness to participate in this process.

At this point, ladies and gentlemen, I would ask each and every one of you to stand. My courtroom deputy will swear you in as prospective jurors. So if you wouldn't mind standing and taking the oath.

THE CLERK: Please raise your right hand.

(Prospective jurors sworn, 9:22 a.m.)

(Jury voir dire reported; not transcribed herein.)

(Jury sworn, 12:07 p.m.)

THE CLERK: Thank you. You can be seated.

THE COURT: Okay. Ladies and gentlemen, I'm going to read to you some preliminary instructions, and I want to apologize for not maintaining eye contact with you in speaking to you directly. I am going to read them. They are written. At the conclusion of the case, you will receive written jury instructions which will govern your duties as jurors and specifically the law that will apply to the facts as you find them in the case.

So you need not take any notes. You do have notebooks, I believe, on your chair. If you don't have them on your chair now, they will be when you return from lunch. These preliminary instructions will take about 10, maybe 15 minutes, and at the conclusion of the instructions, we will break for lunch and then begin with the taking of testimony after the opening statements in the afternoon.

So the preliminary instructions are as follows: You are now the jury in this case, and I want to take a few minutes to tell you something about your duties as jurors and to give you some preliminary instructions. At the end of the trial, I will give you more detailed, written instructions

that will control your deliberations.

When you deliberate, it will be your duty to weigh and to evaluate all the evidence received in the case, and, in that process, to decide the facts. To the facts as you find them, you will apply the law as I give it to you, whether you agree with the law or not.

You must decide the case solely on the evidence and the law before you. Perform these duties fairly and impartially. You should not be influenced by any person's race, color, religious beliefs, national ancestry, sexual orientation, gender identity, gender or economic circumstances.

Also, do not allow yourself to be influenced by personal likes or dislikes, sympathy, prejudice, fear, public opinion or biases, including unconscious biases. Unconscious biases are stereotypes, attitudes or preferences that people may consciously reject but may be expressed without conscious awareness, control or intention. Like conscious bias, unconscious bias can affect how we evaluate information and make decisions.

This is a criminal case brought by the United States Government. The government charges the defendant with distribution of fentanyl resulting in death of another; that is, the individual identified by the initials C.M.R., in violation of Section 841(a)(1) and (b)(1)(C) of Title 21 of

the United States Code.

The charge against the defendant is contained in the indictment. The indictment simply describes the charge the government brings against the defendant. The indictment is not evidence and does not prove anything. The defendant has pleaded not guilty to the charge and is presumed innocent, unless and until the government proves the defendant guilty beyond a reasonable doubt. In addition, the defendant has the right to remain silent and never has to prove innocence or present any evidence.

To help you follow the evidence, I will now give you a brief summary of the elements of the crime that the government must prove to make its case.

First, the defendant knowingly distributed fentanyl. Second, the defendant knew it was fentanyl, or some other federally controlled substance. And, third, the use of the fentanyl distributed by the defendant resulted in the death of C.M.R.

The evidence you are to consider in deciding what the facts are consists of: First, the sworn testimony of any witness. Second, the exhibits that are received in evidence. And, third, any facts to which the parties agree.

The following things are not evidence and you must not consider them as evidence in deciding the facts of this case: First, statements and arguments of the attorneys.

Second, questions and objections of the attorneys. Third, testimony that I instruct you to disregard. And, fourth, anything you may see or hear when the Court is not in session, even if what you see or hear is done or said by one of the parties or one of the witnesses.

Evidence may be direct or circumstantial. Direct evidence is direct proof of a fact, such as testimony by a witness about what that witness personally saw or heard or did. Circumstantial evidence is indirect evidence, that is, it is proof of one or more facts from which you can find another fact.

You are to consider both direct and circumstantial evidence. Either can be used to prove any fact. The law makes no distinction between the weight to be given to either direct or circumstantial evidence. It is for you to decide how much weight to give to any evidence.

There are rules of evidence that control what can be received in evidence. When a lawyer asks a question or offers an exhibit in evidence, and a lawyer on the other side thinks that it is not permitted by the rules of evidence, that lawyer may object. If I overrule the objection, the question may be answered or the exhibit received.

If I sustain the objection, the question cannot be answered or the exhibit cannot be received. Whenever I sustain an objection to a question, you must ignore the

question and must not guess what the answer would have been.

Sometimes I may order that evidence be stricken from the record and that you disregard or ignore the evidence. That means that when you are deciding the case, you must not consider the evidence that I told you to disregard.

In deciding the facts in this case, you may have to decide which testimony to believe and which testimony not to believe. You may believe everything a witness says or part of it or none of it.

In considering the testimony of any witness, you may take into account: First, the witness' opportunity and ability to see or hear or know the things testified to; second, the witness' memory; third, the witness' manner while testifying; fourth, the witness' interest in the outcome of the case, if any; fifth, the witness' bias or prejudice, if any; sixth, whether other evidence contradicted the witness' testimony; seventh, the reasonableness of the witness' testimony in light of all the evidence; and, eight, any other factors that bear on believability.

You must avoid bias, conscious or unconscious, based on a witness' race, color, religious beliefs, national ancestry, sexual orientation, gender identity, gender or economic circumstances in your determination of credibility.

The weight of the evidence as to a fact does not necessarily depend on the number of witnesses who testify

about it. What is important is how believable the witnesses are and how much weight you think their testimony deserves.

I will now say a few words about your conduct as jurors. First, keep an open mind throughout the trial and do not decide what the verdict should be until you and your fellow jurors have completed your deliberations at the end of the case.

Second, because you must decide this case based only on the evidence received in the case and on my instructions as to the law that applies, you must not be exposed to any other information about the case or to the issues it involves during the course of your jury duty.

Thus, until the end of the case or unless I tell you otherwise, do not communicate with anyone in any way and do not let anyone else communicate with you in any way about the merits of the case or anything to do with it.

This restriction includes discussing the case in person, in writing, by phone, tablet, computer, or any other means, via e-mail, via text messaging or any Internet chat room, blog, website or application, including but not limited to Facebook, YouTube, Twitter, Instagram, LinkedIn, Snapchat, TikTok or any other forms of social media.

This restriction also applies to communicating with your fellow jurors until I give you the case for deliberation. And it applies to communicating with everyone

else, including your family members, your employer, the media or press and the people involved in the trial; although you may notify your family and your employer that you have been seated as a juror in the case and how long you expect the trial to last.

But if you are asked or approached in any way about your jury service, or anything to do with this case, you must respond that you have been ordered not to discuss the matter. In addition, you must report the contact to the Court.

Because you will receive all the evidence and legal instruction you properly may consider to return a verdict, do not read, watch or listen to any news or media accounts or commentary, if any, about the case or anything to do with it. Although I have no information that there will be any news reports about this case.

Do not do any research, such as consulting dictionaries, searching the Internet or using other reference materials, and do not make any investigation or in any other way try to learn about the case on your own. Do not visit or view any place discussed in this case and do not use the Internet or any other resource to search for or view any place discussed during the trial.

Also, do not do any research about the case, the law or the people involved, including the parties, the witnesses or the lawyers, until you have been excused as jurors. If

you happen to read or hear anything touching on this case in the media, turn away and report it to me as soon as possible.

These rules protect each party's right to have this case decided on the evidence that has been presented here in court. Witnesses here in court take an oath to tell the truth, and the accuracy of their testimony is tested through the trial process. If you do any research or investigation outside the courtroom, or gain any information through improper communications, then your verdict may be influenced by inaccurate, incomplete or misleading information that has not been tested by the trial process.

Each of the parties is entitled to a fair trial by an impartial jury, and if you decide the case based on information not presented in court, you will have denied the parties a fair trial.

Remember, you have taken an oath to follow the rules, and it is very important that you follow these rules. A juror who violates these restrictions jeopardizes the fairness of the proceedings and a mistrial could result that would require the entire process to start over. If any juror is exposed to any outside information, please notify the Court immediately.

At the end of trial, you will have to make your decision based on what you recall of the evidence. You will not have a written transcript of the trial. I urge you to

pay close attention to the testimony as it is given.

If you wish, you may take notes to help you remember the evidence. If you do take notes, please keep them to yourself until you and your fellow jurors go to the jury room to decide the case. Do not let note-taking distract you from being attentive.

When you leave the court for recesses, your notes should be left in the courtroom. No one will read your notes. Whether or not you take notes, you should rely on your own memory of the evidence. Notes are only to assist your memory. You should not be overly influenced by your notes or those of your fellow jurors.

Following our lunch recess, the next phase of the trial will begin. First, each side may make an opening statement. An opening statement is not evidence. It is simply an outline to help you understand what the party expects the evidence will show. A party is not required to make an opening statement.

The government will then present evidence and counsel for the defendant may cross-examine. Then if the defendant chooses to offer evidence, counsel for the government may cross-examine. After the evidence has been presented, I will instruct you on the law that applies to the case and the attorneys will make closing arguments. And after that, you will go to the jury room to deliberate on

your verdict.

Ladies and gentlemen, that concludes the preliminary instructions.

One housekeeping matter that I would like to alert you to. To your rear, there is a door right there; and that is the jury room. And you are free to use that room throughout your service. There is a refrigerator, some water, some refreshments in there, and you will have access to that during your breaks, the 15-minute break in the morning and the 15-minute break in the afternoon.

If you choose to do so, you may also bring your lunch tomorrow and use that room during the lunch break. But if you have any personal items that you'd rather not carry around, you may leave them in the jury room, and this area will be secure when you're not present in the courtroom.

So with that, ladies and gentlemen, it is 20 past 12:00. We will reconvene at 1:20 to begin opening statements of the parties and then taking the evidence in this case.

I wish you all a very good lunch. And we'll see back in one hour's time. Thank you very much.

(Jury absent, 12:23 p.m.)

THE COURT: Okay. We are outside the presence of the jury. All counsel are present. Mr. Atta is present.

Is there anything we need to take up before we break for lunch?

MR. ROTH: Your Honor, we don't necessarily need to resolve it, but there is one exhibit that with the benefit of further thought, I think it's appropriate to at least flag it for the Court.

Exhibit 43, Your Honor, is a screen capture of a portion of a newspaper article, Associated Press newspaper article that is in the defendant's phone. It's accompanied by Exhibit 44, which is the newspaper article itself redacted, except for the date and the same portion that's in the screen capture.

We intend to offer 43 for its effect on the observer, i.e., what it communicated to the defendant about the possibility of M30 pills as a mark, as a drug to sell in the market, and the possibility to him that they contained fentanyl. Not for its truth, but for notice. Our view is it's a nonhearsay piece of evidence. It's proper. It's admissible, so on and so forth.

When Your Honor reads it, though, it talks about things like the Sinaloa cartel and deaths and things like that. So we would seek the Court's guidance on whether or not any portion of it should be redacted in the Court's view.

THE COURT: Okay. Mr. Cortez.

MR. CORTEZ: We have extremely strong objections to it. I'm not even beginning with evidentiary foundations. How are they going to lay a foundation that my client

actually read it, No. 1?

But more importantly, this is the very type of sensationalist newspaper article that I could come up with that says the federal government is incompetent all the time. That's not permitted. So I have really strong objections to the language in it. It's prejudicial.

So beyond that, Your Honor, I really believe this is the type of evidence that injects error needlessly.

THE COURT: Okay. I will take a look at 43 and 44. Thank you for flagging those.

Will that be admitted or sought to be admitted this afternoon?

MR. ROTH: Unlikely.

THE COURT: Okay. All right. I will take a look at those, and if necessary, we'll have a further conversation. I hope you all enjoy your lunch.

MR. ROTH: Thank you, Your Honor.

MS. FORREST: Thank you, Your Honor.

MR. CORTEZ: Thank you, sir.

(Recess, 12:26 p.m. to 1:22 p.m.)

(Jury present, 1:22 p.m.)

THE COURT: Okay. We are back on the record. All of our jurors are present. All counsel are present. Mr. Atta is also present.

Is the government ready to proceed with your opening

statement?

MS. FORREST: Yes, Your Honor.

THE COURT: Please, go ahead.

MS. FORREST: Okay. I believe I project loudly enough, but you will see the price that lawyers pay if we don't project. She'll interrupt me and ask me to put on a mic.

Now, can everyone see the screen in front of them? And do you see text on that screen? It's a blue screen?

Okay. May I proceed, Your Honor?

THE COURT: Yes, you may.

MS. FORREST: Thank you.

Ladies and gentlemen, this is a photo of Christian Michael Reed. He was born on May 20th, 1994, and he died just one day after his 26th birthday, May 21st, 2020. He was only 26 years old and a lance corporal in the United States Marine Corps, and he died from overdosing on fentanyl-laced pills that this defendant, Nameer Mohammad Atta, sold him.

In the early evening hours of May 21st, 2020, Mr. Reed was in his barracks and he was playing X-Box video games with his friend. Now, shortly after signing off playing that video game with his friend, Mr. Reed was discovered unconscious by his commanding officer.

First responders were on the scene within minutes, and they immediately determined that Mr. Reed had fallen into

cardiac arrest. They will tell you that they did what they are trained to do. They gave Mr. Reed CPR. They gave him medication for cardiac arrest. And they gave him Narcan.

Now, you'll hear that Narcan is a specially designed medication specifically to reverse an opioid overdose. And they'll tell you that they gave him Narcan in case Mr. Reed had fallen into cardiac arrest due to an opioid overdose. Nothing worked. One day after his 26th birthday, Mr. Reed was dead. May 21st, 2020, at 7:19 p.m.

You're going to hear that a medical examiner conducted an autopsy, reviewed Mr. Reed's medical records, and that the medical examiner also reviewed toxicology results of everything that was in Mr. Reed's body at the time of death.

You'll hear that this trained medical examiner, he ruled out any other cause of death, and he determined that fentanyl, a dangerous opioid, killed Mr. Reed. When the medical examiner determined that Mr. Reed died from a drug overdose, special agents launched an investigation to figure out, how did Mr. Reed get those drugs?

You're going to hear that special agents obtained a search warrant for Mr. Reed's phone, and they reviewed communications between Mr. Reed and the defendant on a social media application called Snapchat. Now, these communications between the defendant and Mr. Reed on Snapchat, the numerous

communications, showed that the defendant was discussing selling drugs to Mr. Reed. You're also going to hear that agents were able to find Venmo transactions from Mr. Reed paying the defendant for drugs. So naturally, after finding this, agents got a search warrant for the defendant's phone.

Ladies and gentlemen, the evidence will show that from reviewing Mr. Reed's phone and the defendant's phone, agents were able to confirm that in the last month of Mr. Reed's life, numerous times the defendant sold him drugs, and on May 20th, 2020, Mr. Reed's 26th birthday, the defendant sold him drugs for that last time and those drugs would kill Mr. Reed the very next day.

You're going to hear a lot about Snapchat throughout this trial, and the reason you're going to hear so much about Snapchat is because that was the preferred platform that the defendant used to market and sell drugs to his customers.

You're going to hear terms from special agents like "trap" and "trap house." The special agents will explain to you that "trap" and "trap house," they commonly refer to a place where drugs are sold. You're also going to learn that the defendant repeatedly referred to his apartment as a trap or trap house.

But you're not just going to hear street terms for where drugs are sold, you're going to hear street terms about drugs themselves. You're going to hear terms like Percocet,

Perc 30s, M30s, counterfeit pills, counterfeit percs. But, ladies and gentlemen, the main thing to keep in mind when you listen to all the evidence, is that all of these terms refer to drugs, controlled substances.

Now, throughout this trial, you're also going to hear about some particular evidence agents were able to find on the defendant's phone. Agents are going to tell you that they were able to recover a detailed ledger, a drug sales ledger in the defendant's phone in the Notes application.

They are going to describe to you that the defendant kept detailed records of his drug transactions and that this document was titled "My Own Sales." And that right after that title, "My Own Sales," there is a little emoji with a bag of cash and a dollar sign on the outside.

Ladies and gentlemen, the defendant recorded all of his drug transactions. And you're going to see that these transactions are consistent with every time, every Snapchat communication, every Venmo transaction, every in-person pickup of the drugs that the defendant sold to Christian Reed.

You're also going to hear that the defendant, he didn't just use Snapchat to market drugs and sell drugs to customers like Christian Reed. He didn't just keep a detailed sales ledger of all of his drug transactions. Ladies and gentlemen, you're going to hear that the defendant

recorded his drug sales on video.

You're going to see a video that was retrieved from the defendant's phone, recorded on May 20th, 2020, Christian Reed's 26th birthday that the defendant took of Mr. Reed driving away from his apartment at approximately 4 o'clock in the afternoon.

You'll also see a still from this video that shows there is a distinctive dent in the front of the car that Christian Reed was driving when he went to the defendant's house. You'll be able to confirm for yourselves by seeing the video that the defendant took was of Christian Reed's car because a photo of that same car with that distinctive dent was found on Mr. Reed's phone, and that photo had been taken on May 19th, 2020.

The evidence is going to show that through the defendant's Snapchat communications with Mr. Reed, through the defendant's Venmo transactions with Mr. Reed, through the defendant's detailed drug-sales ledger documenting his drug sales to Mr. Reed, and the very video the defendant took on Christian Reed's birthday, that all of this evidence confirms that the defendant is the person who sold Christian Reed the drugs that would kill him on May 20th, 2020, a little after 7:00 p.m.

At the end of this trial, we will ask that you return a verdict of guilty.

Thank you.

THE COURT: Thank you, Ms. Forrest.

Mr. Cortez, do you wish to make an opening statement?

MR. CORTEZ: Yes, Your Honor. Thank you.

Can we put up the picture again of Mr. Reed? Thank you.

Members of the jury, I hope you can all hear me without a microphone. Thank you.

I heard something interesting in the prosecutor's opening, interesting to me right now, because I know the case. I ask you to keep your mind on this fact that I'm about to mention. She said four times, the defendant, meaning Mr. Atta, a 22-year-old man, sold Mr. Reed drugs. She used the word "drugs."

This case is, indeed, a tragic trial about the death of a young man. You see his picture. But the evidence you will hear as it comes in from witnesses, from documents, from scientific facts, even their witnesses will give you facts, is going to show that every Snapchat, everything that has nothing to do with Mr. Reed is evidence of drugs, as they say, evidence that you will see at the end of this case once the evidence is in, may not help you to decide what the real issue here really is.

Because obviously there are two sides to this story.

And let me see if I can foresee what's going to happen here with the evidence. Yes, on May 21st, 2020, around -- no one knows exactly what time. Sometime around 5:30 in the afternoon, 6 o'clock or so, Mr. Reed started having serious medical problems.

The pathologist who will testify here, Dr. Platt, Dr. Platt will testify from up there and will show you his autopsy findings. He will testify that this young man had a preexisting series of health conditions, all serious.

He will also tell you what he found in the body after they did the autopsy. Members of the jury, please listen for that evidence, keep a very close eye on the science, on the facts, the undisputed facts, and look to see, some of you have training, technical training, look to see what was found in the autopsy.

Let me tell you what the prosecutor mentioned only once -- actually, maybe she didn't. You know the drugs that they are going to try to prove were sold by Mr. Atta. She said fentanyl. Their evidence is going to prove it wasn't fentanyl. It was Percocet. Should that make a difference? You bet. You bet.

Because all the Snapchat with Mr. Reed, he was looking for Percocet tablets, percs, M30's. He had been using them before. This is not the first time. And you'll see he purchased several times, from their own evidence,

several times he purchased Percocets. Not only on May 20. Several times. He had been taking them.

May 21, he's dying. You have several first responders. You will hear them testify, good professionals. You will see that when they got to the scene, two of them specifically, first responders, they started trying to keep Mr. Reed stabilized. One of them intubated him. That simply means to stick a tube in his trachea to give him oxygen. Okay. This same officer will tell you that he smelled alcohol reeking all over the place as he was trying to intubate him. At this point, Mr. Reed was still alive.

Prosecutor tells you, one of them gave him Narcan. Let me tell you how it really happened, how the evidence will show, when Keith Naylor, the paramedic from MCRD testifies, he will testify that he got there with another first responder, Aaron Balderson, Captain Aaron Balderson.

Twenty minutes elapsed. Twenty minutes as Mr. Reed is fighting for his life. So there were medical people there. They could have given him Narcan immediately upon seeing him. We've all heard about fentanyl overdoses. Everybody knows about them.

The Narcan came much later. When Officer Naylor calls in the hospital, because they don't know what else to do, Mr. Reed is slipping away, tragically. With two attendants there to revive him, they call in and want

permission to stop resuscitating efforts. They are going to testify that way. It's denied.

The first request to terminate resuscitation efforts was denied by the hospital. They keep going. Struggling. They make a second -- oh, by the way, at that point, the hospital tells them, Give him Narcan.

More than 20 minutes later, members of the jury, more than 20 minutes later, as Mr. Reed is sitting there dying, look at him, nice, young man. The evidence will prove that he was struggling for a very long time with drug addiction, with pain medication. He was prescribed oxycodone by the doctor themselves a month earlier.

I please ask you to keep an open mind when you listen to the pathologist, the toxicology report that he's going to talk about, and then look for one piece of evidence, because I will, too. Look for a piece of evidence you'll find in the pathologist's report, the autopsy report, and the toxicology report. Look for that piece of evidence that says Percocet, oxycodone. Don't let them shade the truth. Don't let them describe something else or an exception.

Because along with that, you're going to find another piece of evidence that will be undeniable in the case and that you will hear from the medical professionals. What happens to the eye when you're having a fentanyl overdose, an opioid overdose is you look in somebody's eyes as they are

undergoing an overdose, and the pupils of their eyes, they are constricted. They are called pinpoint pupils. Tiny little dot of the pupil you see. That's a classic fentanyl overdose symptom.

Keep a very close eye on the evidence to see how Mr. Reed's pupils were. That's a crucial piece of evidence you will see. And go on the facts, go on the science. Don't go on explanations. Remember, an expert's opinion is an opinion. Look for the evidence that the results of the autopsy prove beyond a reasonable doubt that Mr. Reed's overdose was for fentanyl. You will see some facts that lead away from that.

Then you will also see and hear that only a month earlier, Mr. Reed had been taken to the hospital, a month earlier. He was taken to the hospital because he was suffering what everyone at the time thought was a stroke.

Now, you have some very good professionals, medical people who knew what they were doing, very competent people. They attended to Mr. Reed, and no one diagnosed that stroke as a fentanyl overdose. No one. And medical professionals are trained to recognize those things so they can administer Narcan, so they can administer the proper urgent medical care.

They thought he was going through a stroke. In fact, you will hear from their very witnesses that they did

not think that was a fentanyl overdose. Jump forward to May 21st, then I'll sit down.

May 21st, 2020, Mr. Reed's birthday was the day before. This is a tragedy. There is a victim in this case without a doubt. We don't deny that. He died after he had medical professionals for 20 minutes working on him. Don't give him Narcan until the very end, but it was too late.

Look very closely at what they prove with their evidence about something that has a technical, chemical name. And please remember this word: Metabolites. Metabolites. Metabolites.

Thank you for your time.

THE COURT: Thank you, Mr. Cortez.

MS. FORREST: Yes, your Honor. The United States calls Aaron Balderson.

THE CLERK: Right up here. Please raise your right hand.

AARON BALDERSON, a government witness, having been duly sworn, testified as follows:

THE WITNESS: Yes.

THE CLERK: Thank you. If you can please state your name and spell it for the record.

THE WITNESS: Aaron Balderson, A-a-r-o-n B-a-l-d-e-r-s-o-n.

MS. FORREST: Your Honor, may I proceed?

THE COURT: Yes, please.

MS. FORREST: Thank you.

DIRECT EXAMINATION

BY MS. FORREST:

Q. Good afternoon.

A. Good afternoon.

Q. Can you please pull the mic a little bit closer to you. We have someone taking down everything you say, and we need to make sure she can hear you.

A. Is that better?

Q. Yes, I believe so.

Where do you work?

A. I work at LifeCare Medical Transports in --

THE REPORTER: I'm sorry.

THE WITNESS: I work at LifeCare Medical Transports in Fredericksburg, Virginia.

Q. BY MS. FORREST: And what does your job at LifeCare Transports involve?

A. I'm the Director of Operations, so I supervise all ambulance crews that are staffed and make sure they get their patients picked up and dropped off on time from various locations.

Q. Now, do the individuals you supervise go on calls; is that the terminology?

A. Yes.

Q. And do you, yourself, ever go on calls?
A. Yes.
Q. Under what circumstances do you go on calls?
A. Usually if call volume is high or too high for my staff to handle, or if we're short-staffed or both.
Q. Is that often the case now?
A. At least a couple of days a week, yes.
Q. Are you certified as an EMT?
A. Yes.
Q. And can you explain for the jury what is an EMT.
A. So an EMT is a health-care provider that is trained to provide medical care, both for medical emergencies and injuries, both prior to and during transport to the hospital.
Q. When did you first become an EMT?
A. In April of 2008.
Q. And what did you do to become an EMT?
A. I had to go through 110 hours of classroom instruction, as well as 10 hours in the field for clinicals.
Q. What types of things did you learn in the 110 hours of instruction?
A. We learn anything from, you know, different types of medical emergencies, such as diabetic emergencies, strokes, heart attacks, and how to treat those, as well as traumatic injuries from car accidents, gunshot wounds, falls, how to

treat those, as well as a little bit of EMS operations.
Q. What is EMS operations?
A. Just when you come to a scene, one of the first things you're supposed to look for when to identify and when you need additional resources, how to call for those additional resources.
Q. Okay. Can you briefly explain for the jury what you learned. What are the first things you look for when you arrive on the scene?
A. So if I was to arrive at, say, a car accident, the first thing I want to assess is, is the scene safe? Are there downed power lines? Is there fire? Is there slow-moving traffic that's going to put me and my crew at risk?

The second thing I want to determine is the number of patients that I have and the severity of their injuries; you know, which hospitals are closer to me, what are they capable of? Do I need to call for a helicopter, things like that.
Q. Now, is one of the things that you learn, sort of triaging, learning how you prioritize what you need to do when you're treating someone?
A. Can you repeat your question?
Q. Of course.

Is one of the things that you learn when you get on scene is figuring out what's the most important aspect of the

medical emergency with an individual?
A. Yes.
Q. What you have to treat?
A. Yes.
Q. Okay. In instances of cardiac arrest, what's the most important thing for you to do?
A. Starting chest compression and securing an airway.
Q. And is that to hopefully bring the person out of cardiac arrest?
A. Yes.
Q. Okay. Can you outline for the jury your training and experience as an EMT following 2008.
A. Okay. So following 2008, when I was first certified, I was actually already a member of a volunteer rescue squad. I continued my service with that voluntary rescue squad from 2008 to 2013. From 2009 to 2010, I was a member of a second volunteer rescue squad while I was in college.

In 2013, I went to my first duty station as a Marine, which was in California; so there wasn't really an opportunity for me to volunteer as an EMT. Shortly after that, I went to Okinawa, at which point my EMT certification lapsed. I recertified in 2018 and then joined another volunteer rescue squad in 2018 until 2019, when I was reassigned back to California.

Q. Why did your EMT certification lapse when you were in

Okinawa?
A. I just -- I didn't have the opportunity to volunteer, and I didn't keep up with my continuing education hours, so I had to take the class again.
Q. And so is it accurate to say that when this certification lapsed, you were in the Marines and not working as an EMT?
A. That's correct.
Q. Okay. And when did you get your recertification?
A. 2018.
Q. And have you maintained that certification since?
A. Yes.
Q. Did you attend college?
A. Yes.
Q. Where?
A. Virginia Tech.
Q. What did you study at Virginia --
A. Biology.
Q. Could you give me a moment to make sure I've finished.
A. Yes.
Q. For example, give me a moment to make sure I finish my question, and then you can start your answer so, again, the court reporter gets everything down.
A. Yes.
Q. So it sounds like you studied biology. When did you

graduate?
A. 2012.
Q. And did your biology degree that you obtained in 2012, did that help you working as an EMT?
A. Yes.
Q. How so?
A. I took a lot of medically related classes for my degree, and I also had a concentration in pre-medicine; at the time, I was considering going to medical school. So an example of some of those classes were anatomy and physiology, microbiology.
Q. Now, what does it mean to be a nationally registered EMT?
A. Being a nationally registered EMT means that you have met certain standards and requirements set by the National Registry of Emergency Medical Technicians. It's a national body that just -- it sets a federal or national standard for EMTs, as opposed to differing standards across states.
Q. And when did you first obtain that national certification?
A. 2018.
Q. Okay. And you mentioned serving in the Marine Corps. What time period did you serve?
A. From 2012 to 2022.
Q. And what was your rank when you separated?

A. Captain.
Q. And what was your rank in May of 2020?
A. Captain.
Q. Now, on May 21st, 2020, did you respond to a dispatch call for an unconscious male in his barracks?
A. Yes.
Q. Approximately what time did you receive that call?
A. Approximately 6:30 in the evening.
Q. Now, were you working in your capacity as a Marine or as an EMT when you heard that call from dispatch?
A. I was working in my capacity as a Marine.
Q. Did you respond to the call?
A. Yes.
Q. Why did you respond to the call, even though you were working in your capacity as a Marine?
A. It was close-by, and it was a medical issue for which I'm trained at a higher level than my police officers to intervene and treat.
Q. What did you understand the medical issue to be when you initially received the call?
A. When I initially received the call, I understood it to be a male unconscious in the barracks.
Q. Did you later determine the identity of the unconscious male?
A. Yes.

Q. And what was his name?
A. Christian Reed.
Q. As soon as you got the call, did you head to Mr. Reed's barracks?
A. Yes.
Q. Okay. Did you receive any updates while you were en route?
A. Yes. Shortly after we were initially dispatched and I responded, the dispatcher notified us that witnesses that had found Mr. Reed were reporting that he was gurgling and turning purple.
Q. When you heard from dispatch that there had been reports that Mr. Reed was gurgling and turning purple, given your training as an EMT, what did that mean to you?
A. To me, it meant that it was very likely that he was in cardiac arrest.
Q. Why did you think that Mr. Reed was in cardiac arrest?
A. The turning purple is consistent with a lack of oxygen to the body. I mean, you know, some people call it blue; that's typically what you hear. But just the discoloration overall as purple or blue indicated to me that he was starved for oxygen, and the gurgling the witnesses were reporting sounded to me like agonal respirations.
Q. And what's "agonal respirations"?
A. Agonal respirations are respirations that a person takes

shortly after they pass away, where it's not really an effective breath. It's more of a reflex than anything else.

Q. There should be a binder in front of you. Do you see a large binder --

A. Yes.

Q. -- that has multiple tabs?

Can you please place that in front of you and take a look at Tabs 83 and 84. Just let me know when you've had a moment to review those tabs.

A. I have.

Q. Okay. What do you see behind Tabs 83 and 84?

A. I see photographs of Barracks 5115. Looks like the section at the barracks containing rooms 25 to 34.

Q. Are these a fair and accurate representation of Mr. Reed's barracks?

A. Yes.

MS. FORREST: Your Honor, the United States moves Exhibits 83 and 84 into evidence.

MR. CORTEZ: No objection.

THE COURT: 83 and 84 are received.

(Government's Exhibits 83 and 84 received in evidence.)

MS. FORREST: Please pull up Exhibit 83.

Q. Okay. Now, you said that you received the call sometime around 6:30. After receiving the call, being en route,

getting updates, approximately how long did it take you from hearing the dispatch call to getting to Mr. Reed's barracks to treat him?

A. It was less than five minutes.

Q. Okay. Now, can you tell us what we're looking at here with Exhibit 83.

A. This is a photo of Mr. Reed's barracks room. He was on the first floor, so you would go into that open doorway there to the left of the photo and immediately turn right adjacent to the stairwell to enter his room.

Q. Okay. We can take it down. Thank you.

When you arrived to Mr. Reed's barracks, what did you see?

A. I saw my police officers beginning CPR. One of them was performing chest compressions, and the other one was attaching the AED.

Q. Now, did firefighters and paramedics ultimately arrive to help with your treatment of Mr. Reed?

A. Yes.

Q. Approximately how long after you got on scene did firefighters and paramedics arrive to assist?

A. It was less than two minutes.

Q. Okay. Now, earlier you said you thought that Mr. Reed was in cardiac arrest based on the updates you had received from dispatch. When you got to Mr. Reed's room, did he, in

fact, appear to be in cardiac arrest to you?
A. Yes.
Q. Did you know why Mr. Reed was in cardiac arrest?
A. No.
Q. Did you try to determine why Mr. Reed was in cardiac arrest?
A. Not initially, no.
Q. Why not?
A. When you're responding to someone that's in cardiac arrest, that's not really -- that's not what you do. You immediately start chest compressions and you try to secure an airway to try and start saving the person's life and get them out of cardiac arrest and restore a normal rhythm to their heart.
Q. Is it standard in cases of cardiac arrest that your top priority is doing whatever you can to get the person out of cardiac arrest, rather than --

MR. CORTEZ: Objection. Leading.

THE COURT: Overruled. You may ask the question.

MS. FORREST: Let me restate it.

Q. Is it standard practice in cases of cardiac arrest to make your top priority getting the patient out of cardiac arrest rather than trying to spend time figuring out what put them in cardiac arrest?
A. That's correct.

Q. Okay. Now, did you have any guesses as to why Mr. Reed was in cardiac arrest?

MR. CORTEZ: Objection. Speculative.

THE COURT: Sustained.

Q. BY MS. FORREST: Okay. Now, at some point when you entered Mr. Reed's barracks, did you have a body camera on?

A. Yes.

Q. Did you at some point turn that body camera on to start recording?

A. Yes.

Q. And why did you do that?

A. It was our policy to activate body cameras on calls. I also believed it would be a helpful tool, both for investigative purposes as well as for training purposes. I worked right next to the fire chief, and he and I had a pretty good relationship, so I thought it would be beneficial for him and his personnel as well.

Q. And when in relation to when you entered Mr. Reed's barracks did you turn your body camera on?

A. It was shortly after I entered. I believe it was once I approached my officers to assist them, I turned it on at some point then.

MS. FORREST: Now, would you take a look in the binder, I believe in the sleeve you'll see some discs. The discs are numbered 203 and 204. They have been marked for

purposes of identification.

One moment, Your Honor.

Q. Have you had a chance to look at them?

A. I see the two discs I initialed. I just don't see the 203 or 204.

Q. Okay. But do you see that your initials are on both of these discs?

A. Yes.

Q. And for one of the discs, which we can -- we will mark as 203 for purposes of identification, was that the full body-camera footage from when you started recording when you got in Mr. Reed's barracks to when you stopped recording?

A. Yes.

Q. And what we'll mark as Exhibit 204, did you have an opportunity to review clips, so sections taken from that full body-camera footage?

A. Yes.

MS. FORREST: Your Honor, the United States has marked the clips as Exhibits 85, 86 and 87, and we would offer them into evidence.

MR. CORTEZ: May I have a moment, please?

We need to approach.

MS. FORREST: Your Honor, I'm not sure whether the defense has an objection, but I believe we may need to approach based --

MR. CORTEZ: Based on your ruling earlier.

THE COURT: Okay. 85, 86 and 87 have been offered into evidence.

Do you have an objection to those, Mr. Cortez?

MR. CORTEZ: I do, sir.

THE COURT: Okay. The objection is sustained.

Q. BY MS. FORREST: Okay. When you arrived to Mr. -- when you arrived to Mr. Reed's barracks, what type of care did you see being administered to him?

A. When I first got there, one of my police officers was performing chest compressions. The other was attaching AED pads to Mr. Reed's chest.

Q. What is an AED pad?

A. An AED is an automated external defibrillator, and the pads are adhesive pieces that attach to a patient's chest. The AED is designed to deliver electricity to a person's heart in an effort to get it into a normal rhythm.

Q. And is this one of the ways of treating someone who is in cardiac arrest, trying to get them out of cardiac arrest?

A. Yes.

Q. Okay. And I believe you said that you saw chest compressions being performed. Were they being performed by an individual?

A. Yes.

Q. And are chest compressions a way to -- a method of treating someone in cardiac arrest?
A. Yes.
Q. And is the purpose of this trying to get their heart going?
A. The purpose of it is more to manually pump their heart until a normal rhythm can be restored, in an effort to continue to circulate blood throughout the body.
Q. Can you take a look in your binder at tab -- it should be Tab 2. And when you've had a moment, please tell me when you're done.
A. Okay.
Q. And what is Exhibit 2?
A. 2 is a clip of my body-cam footage, showing one of the firefighters rendering aid to Mr. Reed.
Q. And to be clear, when you use the word "clip," I just want to make sure the record is clear, are you referring to a still image from a clip, a photograph?
A. Yes, ma'am.
Q. And is that a photograph from the video footage that your body camera recorded?
A. Yes.
Q. And is it a fair and accurate representation?
A. Yes.

MS. FORREST: Your Honor, the United States offers

Exhibit 2 into evidence.

MR. CORTEZ: Per the Court's -- subject to the Court's ruling, no objection.

THE COURT: Okay. 2 is received.

(Government's Exhibit 2 received in evidence.)

Q. BY MS. FORREST: Now, can you orient us and tell us what we're looking at here.

A. I believe I was looking at a different photo.

Q. Can you turn to Tab 1. I think that may be what you're referring to.

A. Yes. I'm sorry. I was looking at Tab 1.

MS. FORREST: My apologies, Your Honor. I'll correct the record. Exhibit 1 should be admitted into evidence based on Mr. Balderson's testimony. We'll call it Exhibit 2, Your Honor; but it's Tab 1 in the witness' binder.

THE COURT: Okay. Mr. Cortez, do you stipulate that the foundation that was laid for 1 equally applies to 2?

MR. CORTEZ: I don't know.

THE COURT: Okay.

MR. CORTEZ: I have Tab 2, Exhibit 2. Under -- no.

THE COURT: Okay. Ms. Forrest, will you please lay the foundation. 2 is withdrawn --

MS. FORREST: Of course, Your Honor.

THE COURT: -- for lack of foundation.

MS. FORREST: Of course, Your Honor.

Q. Please take a look at the exhibit behind Tab 1.

A. Yes, ma'am. The exhibit behind Tab 1 is a still shot from my body-worn camera that shows one of the firefighters rendering aid to Mr. Reed.

Q. And what is behind Tab 2, what still shot?

A. Behind Tab 2 is another still shot from my body-worn camera that shows me providing artificial ventilations to Mr. Reed.

Q. Okay. Is that the image that was shown on screen?

A. Yes, ma'am.

Q. Okay. And is that a fair and accurate representation from your body-camera footage?

A. Yes, ma'am.

MS. FORREST: Appears there was some confusion, Your Honor, but the United States offers Exhibit 2, which is behind Tab 2 into evidence.

THE COURT: Any objection?

MR. CORTEZ: Not in the Court's ruling, but I just want to make sure we're talking about the same thing. Tab 2, J'me.

MS. FORREST: Yes. My apologies.

MR. CORTEZ: Thank you.

MS. FORREST: And, Your Honor, for the record, as Mr. Cortez has pointed out, it's time stamped 18:53:47.

Q. Is that correct, Mr. Balderson --
A. Yes, ma'am.
Q. -- that that's the time stamp?
THE COURT: Okay. 2 is received.
MS. FORREST: Thank you, Your Honor. Can we please publish it.
Q. Now, can you -- yeah, just orient us and tell us what we're looking at here, where you're standing and what's going on.
A. So I'm at Mr. Reed's head. I'm providing artificial ventilations to him with a bag-valve mask. I also see the AED pad attached to his right chest, as well as the LUCAS, which is a chest compression machine, for lack of a better term, providing chest compressions to Mr. Reed. I also see two IV tourniquets, one on each arm, but I don't see IV access established yet.
Q. Okay. Now, let's start with you. You're trying to provide artificial ventilation I believe is what you said?
A. Yes, ma'am.
Q. What is that?
A. Artificial ventilation is essentially breathing for a patient. I'm using a bag-valve mask, which has a one-way valve on it. When I squeeze the bag that's in my right hand in this photo, it delivers air through a mask, or if there is an endotracheal tube present, which there isn't in this

photo, but it delivers oxygen to Mr. Reed artificially.
Q. And is this another step in an attempt to try to get Mr. Reed out of cardiac arrest, to get him breathing on his own?
A. Yes.
Q. Okay. You referred to the AED pad. Is that the electricity that you mentioned before being delivered to Mr. Reed's chest?
A. It's the pad that would deliver the electricity, yes, ma'am.
Q. Thank you for the clarification.

Earlier you said that chest compressions were manually being delivered at one point when you walked into the barracks to Mr. Reed. Can you explain for the jury what the LUCAS machine is.
A. So the LUCAS is a device that -- it is very physically taxing to deliver chest compressions for an extended period of time. The quality is also a little bit reduced. The LUCAS is a -- it's an acronym -- I don't remember what it stands for. But it's a device that we can apply to a patient that essentially has a -- as you can see, a large plunger in the center of their chest, and that will deliver chest compressions continuously for us.
Q. And is one of the purposes to deliver not only steady compressions, but consistent compressions from the machine?

A. Yes, ma'am.
Q. Is this also in an effort to get Mr. Reed's heart beating again?
A. It's more to circulate blood throughout his body artificially until we can restore his heart to a normal rhythm.
Q. And is that a method of addressing someone in cardiac arrest?
A. Yes, ma'am.
Q. You referred to the IVs -- or not IVs, but I believe IV tourniquets to establish some sort of vascular access. What's the purpose of an IV tourniquet?
A. The purpose of an IV tourniquet is to essentially reduce blood flow away from, in this case the arms, so that veins become easier to access so that you can put an IV in that vein.
Q. And what does the IV itself do?
A. The IV is the path by which paramedics would deliver certain drugs to assist in the treatment of the cardiac arrest.
Q. So cardiac arrest medication?
A. Yes, ma'am.
Q. What are some different cardiac arrest medications?
A. Epinephrine and Atropine are two that I know of.
Q. And do you remember whether cardiac arrest medication

was administered, in fact, to Mr. Reed?
A. I remember, after watching my body-camera footage.
Q. Okay. When you were in Mr. Reed's room, did you see any prescription medication?
A. Eventually, yes.
Q. Okay. Where did you see that medication?
A. I saw it on the -- I believe it was the shelf to the left. Right after you come in the door, there is a little bookshelf to the left, and it was sitting on that.
Q. Would you take a look at Exhibit 88. It should be behind Tab 88. Please let me know when you've had a moment to look at it.
A. Behind Tab 88 is a photo of Mr. Reed's bookshelf that has a prescription pill bottle, cell phone, wallet and some other personal items.
Q. And is the photo behind Tab 88, so Exhibit 88, is that photo a fair and accurate representation of some of the personal items that you observed in Mr. Reed's room?
A. Yes.

MS. FORREST: Your Honor, the United States offers Exhibit 88 into evidence.

MR. CORTEZ: No objection.

THE COURT: 88 is received.

(Government's Exhibit 88 received in evidence.)

MS. FORREST: Can you please publish. Thank you.

Q. What did you do when you saw this prescription pill bottle?
A. I looked at the label to see what the medication was, whose name was on it, when was it filled, how many are left.
Q. And whose name -- let's start with the name.
Whose name did you see on this pill bottle?
A. Christian Reed.
Q. And what did you observe as far as the pills within the pill bottle?
A. It was about a third of the way full. And there were different types of pills in there. It was not all the same pill. Some were caplets, some were tablets and gel caps. I also noticed that it was either filled that day or the day before.
Q. Do you recall whether you opened the pill bottle and looked inside or just observed it from the outside?
A. I don't really recall.
Q. Okay. The cell phone. How -- when you -- is that the state in which you found Mr. Reed's cell phone?
A. Yes, ma'am. That's the state in which it was handed to me.
Q. Okay. Can you explain that to the jury.
A. Shortly after the hospital authorized EMS personnel to cease resuscitative efforts, one of the firefighters on the

scene handed me this phone, telling me that it was open, mentioning something about maybe there is text messages, or something like that. So I took it from him and just sat it down and told him something to the effect of, We'll give that to CID when they get here.

Q. What is CID?

A. It's the Criminal Investigative Division.

Q. Did you go in the phone or do anything with it?

A. No.

Q. So after it was handed to you, you simply placed it on the shelf?

A. Yes, ma'am.

MS. FORREST: Okay. I have no further questions for the witness, Your Honor. Thank you.

THE COURT: Okay. Thank you.

Cross-examination.

MR. CORTEZ: Yes. Thank you.

CROSS-EXAMINATION

BY MR. CORTEZ:

Q. Captain Balderson, good afternoon, sir.

A. Good afternoon.

Q. Let me go back and cover a little more detail about the -- just the last question right now. There was -- you were asked whether -- you mentioned the hospital.

A. Yes.

Q. As you're trying to determine what's going on, you mentioned the hospital giving someone direction; right?
A. Yes, sir.
Q. Now, you had arrived with a colleague; correct? You were not the only one there at the scene; right?
A. I was not the only one at the scene, no, sir.
Q. And besides you attending to -- because I noticed in that picture, one of the pictures, Mr. Reed's outstretched arm and the knees of a person with olive pants; correct?
A. I'm not sure. Can I see the photo again?
Q. Sure. Can we bring it up again, please. Tab 2.
Do you see those pants to the left, top left of the picture?
A. Yes, sir.
Q. Okay. Mr. Reed's arm stretched out?
A. Yes, sir.
Q. Who is that officer?
A. That is a firemen, but I don't know who it is.
Q. And you say it's a fireman because you see the bottom of the jacket they wear, the yellowish --
A. Yeah. The bottom of the jacket and the pants, yes, sir.
Q. Yes, sir. Then there is an arm right in the middle of the picture, transecting the picture. Is that your arm, sir?

A. With the black watch and the rubber glove?
Q. Yes.
A. Yes, sir, that's my arm.
Q. And then I see another hand on the right side of the picture touching Mr. Reed's other arm; right?
A. Yes, sir.
Q. Who would that be?
A. I don't know, sir.
Q. Okay. Were there more than two other people besides you at that time?
A. Yes, sir.
Q. And let's see if we can get this developed a little better.

There is a time stamp and a date stamp on that photograph; right?
A. Yes, sir.
Q. The date stamp is 5/21/2020; correct?
A. Yes, sir.
Q. And military time, 18:53:47; right?
A. Yes, sir.
Q. In civilian time, that would be 6:58 p.m.?
A. 6:53 p.m., yes, sir.
Q. Sorry. 6:53?
A. Yes, sir.
Q. My mistake. Thank you.

And you said on direct examination that you arrived there at 6:30?

A. It was approximately 6:30, yes, sir, is when I was dispatched.

Q. I understand. And you had been there roughly, fair to say, 15, 20 minutes?

A. Yes, sir.

Q. And on direct, you also testified that you were trained to, at the time, determine what is the most important approach to a patient that needs critical care; right?

A. Yes, sir.

Q. It is your job to determine what is going on with the patient; right?

A. To an extent, yes, sir.

Q. Yes. Because you have to administer the right medical aid; right?

A. Yes, sir.

Q. Okay. And we heard about all your training. How many fentanyl overdoses had you experienced in your career?

A. Fentanyl specifically, I don't know.

Q. Roughly, sir.

A. Opioid in general, probably a dozen or so.

Q. A dozen or so. And in your camera -- strike that.

You were interviewed over the weekend by the government, right, for your testimony?

A. Yes, sir.
Q. And in that interview you had, you were asked about what your camera, the body camera that we're talking about that took that still picture, what your camera showed about Mr. Reed's eye pupils; correct?
A. Yes, sir.
Q. And those eye pupils that the camera taped were dilated; right?
A. Yes, sir.
Q. Not pinpoint? Not constricted; correct?
A. That's correct, sir.
Q. Now, when you arrived, because of your medical training and the advanced symptoms you were told, like gurgling, you concluded a cardiac arrest; right?
A. Yes.
Q. Did you look -- when you arrived first and see Mr. Reed, did you look at his eyes?
A. No, sir.
Q. You didn't look at his eyes?
A. Not at first, no, sir.
Q. Right. You were doing all this other stuff for maybe 15, 20 minutes to try to keep him alive; right?
A. Yes, sir.
Q. Okay. And at some point, you got to the point where -- strike that.

Incidentally, when you got there, when you first arrived in front of the barracks, actually on scene, you didn't exactly know where the specific room was; correct?

A. I had trouble finding the room, yes, sir.

Q. You were in a hurry and you were concerned; right?

A. Yes, sir.

Q. And so you approached a duty officer that was sitting there to help people like you; correct?

A. Yes, sir.

Q. And that officer appeared to be not concerned; correct?

A. Yes, sir.

Q. To the point where it irritated you; correct?

A. Yes, sir.

Q. Because he wasn't taking the matter seriously; correct?

A. Yes, sir.

Q. He had to look at a map, he told you?

A. Yes, sir.

Q. And you didn't have time to look at a map; right?

A. No, sir.

Q. So once you realize where you had to go, you rushed in there, you go into the room. Were you the first one in, sir?

A. No, sir.

Q. Who was first in?
A. My two police officers.
Q. I'm sorry?
A. Two of my police officers.
Q. Their names, sir?
A. Officer Holguin and Corporal Vargas.
Q. And what were they doing when you got there?
A. They were initiating CPR on Mr. Reed.
Q. Had they looked at his pupils?
A. I don't know, sir.
Q. So then you begin to administer the aid that you provided. And after time elapsed, one of the people there -- I believe a sergeant -- suggested that you all stop your efforts to keep Mr. Reed alive; right?
A. Can you repeat your question, sir?
Q. Yes. At some point after you tried to restart the cardiac rhythm --
A. Yes.
Q. -- and you were doing oxygenation on the body, one of your officers, or a person there basically said, Let's stop, or this is not working, something like that; correct?
A. No, not really, sir.
Q. Someone had to call the hospital for direction; right?
A. Yes, sir.
Q. Who was that?

A. I believe that would have been EMS Chief Matt Rios. EMS Chief Matt Rios.

Q. Rios. Officer Rios.

What did he say, if anything?

A. I don't really know. I kind of heard it going on in the background, but I wasn't really paying attention to the radio traffic at the time.

Q. He said something; right?

A. I don't remember.

Q. You don't remember specifically what he said?

A. Correct.

Q. Okay. But he said something?

A. Yes, sir.

Q. Okay. And then from whatever he said, someone called the hospital, Sharp Hospital; right?

A. I think -- I thought it was Scripps La Jolla.

Q. Scripps. I'm sorry. I get confused between hospitals. You're right. Scripps La Jolla.

And please tell the jury why he had to call Scripps La Jolla.

A. So once -- once you work a cardiac arrest patient for a certain amount of time to no avail, the senior healthcare provider on scene at the time, which was Mr. Rios, has the ability to call the hospital and speak to a doctor and request permission to cease resuscitative efforts.

Q. And was that the first request to cease resuscitative efforts, or was it the second one?
A. I'm not sure which one you're referring to, sir. But he did ask twice.
Q. Thank you. So there were two requests?
A. Yes, sir.
Q. The first one was denied; correct?
A. Yes, sir.
Q. The second one was granted after they told you to put Narcan into Mr. Reed; right?
A. Yes, sir.
Q. That was over 20 minutes later after you arrived, roughly; correct?
A. Yes, sir.
Q. From your training and medical background, had you identified a fentanyl overdose when you arrived, and you had given Mr. Reed Narcan, maybe it would have helped; correct?

MS. FORREST: Objection. Calls for speculation.

THE COURT: Sustained.

Q. BY MR. CORTEZ: For about 20 minutes, no one, not one of you, gave Mr. Reed Narcan; correct?
A. Yes, sir.

MR. CORTEZ: I have no other questions.

THE COURT: Okay.

MS. FORREST: Your Honor, with the Court's

permission, may this witness be excused?

THE COURT: Yes. Ladies and gentlemen, we are going to take an early afternoon break. There is a legal matter that I must discuss with the attorneys at this time. So we are going to take our 15-minute afternoon recess at this time.

I would admonish you -- and I will give you this admonishment every time we take a recess in this case -- not to form or express an opinion about anything having to do with this case, and not to do any independent research, whether it be on the Internet, books, if people still use books, or anything along those lines. All of the evidence that you will properly consider will be introduced here in court.

So it is 2:25. We'll call it 2:25. Please be back ready to go at 20 till 3:00. Thank you.

And you can leave your notebooks here in the courtroom. And you can go to the jury room, but if you go to the jury room, you have to stay there the entire 15 minutes.

Can we ask someone to shut that door, please.

(Jury absent, 2:24 p.m.)

THE COURT: Okay. We are outside the presence of the jury. All counsel are present. Mr. Atta is present.

Yes, Mr. Cortez.

MR. CORTEZ: Can we have Captain Balderson not

totally excused yet?

THE COURT: Well, we're going to discuss something real quick.

You can step down, sir.

THE WITNESS: Thank you, sir.

THE COURT: Okay. Government wish to be heard?

MR. ROTH: No. I'm waiting for the Court.

THE COURT: Okay. My in-limine ruling regarding the video footage was based upon the state of the evidence as I understood it to be.

Mr. Cortez, during your opening, you made repeated references to the way in which the responding personnel treated C.M.R. And I now believe that based on your cross-examination of this witness, when coupled with the opening that was given, you have made relevant the video footage that was shot by the body camera, because you called into question their actions once they responded to the area, in that they did not administer Narcan.

I think that the jury is properly entitled to see what they did do and the circumstances under which they were making those decisions. So the balancing test that I previously referenced, the balancing equation has shifted somewhat given the opening and given the cross-examination.

Do you wish to be heard?

MR. CORTEZ: I completely agree.

THE COURT: Okay. So the in-limine ruling as to Government's in-limine Motion No. 7 is -- tentatively it was denied. I allowed Government's Exhibit No. 2 to be received in evidence. Government Exhibit 1 not to be received into evidence.

My ruling at this point, given the state of the evidence, is that the video footage shot from the body cam may be introduced in its entirety or in the clips that the government has proposed.

MR. ROTH: Thank you, judge.

MS. FORREST: Thank you, Your Honor.

THE COURT: Okay. So that's the legal -- and I wanted to do that before the witness was excused.

MR. CORTEZ: Absolutely.

THE COURT: And I apologize for the early break.

MR. CORTEZ: No, no. I got it. Absolutely.

THE COURT: But I think --

MS. FORREST: Thank you for clarification.

THE COURT: I think the parties knew where I was going with that.

MR. CORTEZ: Totally agree.

THE COURT: Is there anything else that we need to address before we take a ten-minute recess?

MR. ROTH: I would just observe, Your Honor, that based on the opening, it's the government's position that

Exhibit 43, the screen shot that we referenced before the lunch hour, should now be admitted without any redactions.

Mr. Cortez opened by arguing that there is a difference between fentanyl and Percocets. That's not true as a matter of law, but he has now placed directly in issue the issue of the content of M30 pills, which that screen grab speaks to.

MR. CORTEZ: I don't agree. We're talking about fentanyl.

THE COURT: Okay. I'm paying very close attention to everything that's being said. I'm taking very detailed notes. I did not get a chance over the lunch break to take a look at 43 and 44, so I'm going to reserve ruling on those particular exhibits at this time.

MR. ROTH: Thank you, Judge.

THE COURT: So with that in mind, I leave it up to the parties, the government in particular, whether on redirect you would like to go into the video footage or whether you want to re-call this witness. I know that it's a shifting landscape.

MS. FORREST: Yes, Your Honor.

MR. CORTEZ: Yes, it is.

THE COURT: Okay. We'll be in recess until 20 till.

MR. ROTH: Thank you, Judge.

MS. FORREST: Thank you, Your Honor.

MR. CORTEZ: Thank you.

(Recess, 2:27 p.m. to 3:40 p.m.)

THE COURT: We are back on the record. All jurors are present. All counsel are present. Mr. Atta is present.

I remind you, sir, that you're still under oath.

Okay. Ms. Forrest.

MS. FORREST: Your Honor, the United States does not have any further questions for this witness. And with the Court's permission, may he be excused?

THE COURT: Okay. You may step down, sir.

THE WITNESS: Thank you, sir.

MS. FORREST: Your Honor, the United States calls as its next witness, Keith Naylor. That's with a "Y."

THE CLERK: Thank you. Please raise your right hand.

KEITH NAYLOR, a government witness, having been duly sworn, testified as follows:

THE WITNESS: I do.

THE CLERK: Thank you. If you can, when you're seated, state your name and spell it for the record.

THE WITNESS: Keith Naylor. K-e-i-t-h N-a-y-l-o-r.

MS. FORREST: May I proceed, Your Honor?

THE COURT: Yes, you may.

MS. FORREST: Thank you.

DIRECT EXAMINATION

BY MS. FORREST:

Q. So we have someone taking down everything you say, so I'd ask that you keep the microphone close to you and speak loudly so she can get everything.

Where do you work?

A. Currently, I'm a firefighter/paramedic for the Lakeside Fire Protection District in Lakeside.

Q. What does your job involve on a daily basis?

A. My job involves any manner of 911 responses, primarily -- a majority of our responses are medical aids, approximately 90 percent of those, and anything else that people call 911 for: Vehicle accidents, fires, rescues, anything.

Q. How long have you had your current job?

A. I've been with the Lakeside Fire Protection District for currently about two months, and prior to that, I was with MCAS Fire Department for about 15 years.

Q. Can you say the last part one more time.

A. Yes. I was with MCAS Miramar Fire Department for approximately 15 years.

Q. Now, as a general matter, can you explain to the jury the difference between a paramedic and an EMT. And I'll ask you to slow down a little bit so --

A. Sure.

Q. -- she can get down everything.
A. Yes. EMT, that is -- EMT basic is generally where you'll start your training at. It's basic life support skills, CPR, wound care, basic airway management. There is really not a lot of medications or anything that we'll do as far as evaluating the condition of a patient's heart.

As a paramedic, our scope of practice, as we call it, gets expanded greatly. It's quite a bit more training. We're able to do vascular access, access through -- called intraosseous to the bone.

Then we also go through extensive cardiology training, where we learn how to read EKGs, interpret conditions of the heart.

THE REPORTER: I'm sorry.

THE COURT: If you can just slow down just a little bit.

THE WITNESS: No problem. I know I can talk fast sometimes. I apologize.

THE COURT: And especially with the technical terms, nice and slow.

THE WITNESS: Understood, sir.

In addition to that, we go through a lot of pharmacology training, where we get trained on certain medications that we can use to help treat life-threatening conditions, problems with the heart, problems with the lungs,

metabolic issues. We also are trained in advanced airways, where we can place breathing tubes down people's throats, into their lungs. We have a couple different routes and techniques for that procedure.

So generally, the foundation of an EMT is the same as a paramedic. It's just that a paramedic has extensively more training to perform more invasive procedures, as we call it.

Q. BY MS. FORREST: Can you describe what you had to do to become a paramedic to get to where you are now.

A. Sure. To even apply for paramedic school, you have to be an emergency medical technician basic. That's a relatively short course. It's approximately one semester. When you're completed with that, then you begin kind of your real-world experience, as far as treating patients.

Generally, the paramedic schools want you to have a year of experience. In addition to that, you have to take a few prerequisite classes for paramedic school: Anatomy and physiology, basic pharmacology, a few other involving -- a handful of classes are in there.

To be a paramedic is very much like applying for a job. You have to go through an interview. It's a written interview, maybe a skills exam. It's a whole application process just to get admitted into paramedic school. From paramedic school, we go through about a one-year didactic

program, and that's where we learn all of the -- all the literature, and that's the foundation of our skills.

When you're done with the didactic portion, then we'll be placed into an emergency room, and generally there we'll spend approximately two months. And that's how we learn to assess patients at a more advanced level, learn all of our physical skills, as far as starting IVs and our osseous access, inserting the breathing tubes, as I mentioned before, and getting a real-world experience as far as how the pharmacology we perform actually acts on a patient.

When that's done, then we go out to what we call a field internship. And a field internship is typically about three months, and you're riding with an experienced senior paramedic, and he evaluates all the skills, all the techniques that you've learned over the course of your experience leading up to that and makes a determination whether or not you're going to be a qualified paramedic.

For that, there is a national exam that we take. It's the National Registry of EMTs/paramedic. And then when you get hired with your respective agency, then you usually spend about three months in a field training program. So you're already a paramedic, you're a licensed paramedic, but it's a more technical version of your internship, and that's really where your agency is going to decide if you're someone they want working for you to provide care.

Q. Now, among paramedics, is there what's called a lead paramedic?

A. There is. It depends. It varies from agency to agency. Typically, it will be the paramedic that's assigned to the ambulance that day. It's just kind of a standard that whoever is the paramedic on the ambulance will generally be the lead paramedic for those incidents.

Frequently, we have paramedics assigned to the fire engines that respond with us as well, and oftentimes there can be multiple paramedics there. But there is always a lead. There is always somebody in charge.

Q. So of the people who have gone through the extensive background that you've outlined to become a paramedic, is a lead paramedic often someone who has significant experience even compared to other paramedics?

A. All paramedics are qualified to the same standard. In certain situations, if we encounter a patient that, you know, is uncommon or very technical, then generally the more experienced paramedic will assume patient care and control of that call.

But as far as specifics skill sets or certain advances in the paramedic field, it's -- it's not really commonplace. Every paramedic should be qualified to run every incident. But like I said, sometimes there are situations that arise where you just haven't seen it before

as a new medic and you rely on your more experienced medics to help you get through that.

Q. In all, how many years have you worked as a paramedic?

A. Twenty years. And then two years as EMT basic prior to that.

Q. Now, has your training and experience as a paramedic involved identifying and treating overdoses?

A. Absolutely.

Q. What about opioid drug overdoses?

A. Yes, ma'am.

Q. Can you tell the jury a little bit, a little bit about your experience in identifying and treating drug overdoses.

A. Sure. It comes in all manners, anywhere from the very -- the overdose was witnessed. We can arrive on scene and we have firsthand accounts from the bystanders on the scene saying, yes, this individual suffered an opioid overdose. Here is what it was. Those are very straightforward. We know this is the problem.

There is other instances where we come across a patient who is just nonresponsive and we have to work through an organized assessment program to try and determine what that cause may be. And then there is also instances where we find out that an individual had suffered that overdose, and it's not something that we're aware of until much, much later.

Q. I believe you used the termed "an organized assessment program." What is that?

A. So one of the foundations of all paramedic training. It's very easy, especially as a new paramedic, to arrive on scene, it's very chaotic. Oftentimes there is family. There is bystanders. It can be a very hectic scene.

And if you don't have an organized, methodical approach to your assessment -- basically there is a specific line of questionings or a specific set of tasks that you perform initially -- then it's easy to kind of get derailed, your call will not go smooth. Something that we always train -- that I've always trained my new paramedics with is just being very organized, very detailed and methodical and staying to a strict pattern.

Q. In your experience in treating drug overdoses, including opioid overdoses, have you treated people who were in cardiac arrest due to an overdose?

A. Yes.

Q. And in those instances, what is your top priority when you're treating them, when you start treating them?

A. So if we encounter a patient that's in cardiac arrest, we work under guidelines set forth by the country. It's called advanced coronary life support, and it's a specific set of protocols that we follow. Generally, the initial treatment is more to try and restart the heart, to put it

very plainly.

Until we kind of get the care going and get track of them potentially breathing again, a lot of the other treatments kind of fall secondary and tertiary to that. So our primary role is to try to address the cardiac issue that may be secondary to the primary cause of that.

Q. And why do the secondary concerns, why do they fall -- why do they become secondary and your focus is on treating the cardiac arrest and getting the patient out of that?

A. It's standard protocol.

Q. What are some of the classic signs of an opioid overdose?

A. So with opioid overdose is typically we'll see respiratory depression, so lower respiratory rates. We'll see a decrease in the patient's responsiveness. They may not be alert. They may not be breathing. Oftentimes, if we can encounter that individual early on in the assessment, with what we call pinpoint pupils, there is certain modes of reactions that occur that cause that. But as that arrest continues, some of those signs may become a little more vague.

Q. What do you mean, some of those signs may become a little more vague?

A. Typically, if there is nobody on scene to tell us this is what happened, then our nexus of suspicion, you know, we

tend to fall away from what may have caused that arrest in order to just address that arrest.

So individuals who have been in cardiac arrest for some time, regardless of the cause, those pinpoint pupils may not be evident. Sometimes the pupils become fixed and dilated. That generally occurs a little bit later after the arrest.

Q. So regardless of whatever the cause is of someone falling into cardiac arrest, why is the focus on treating the cardiac arrest?

A. Well, until we get the heart restarted, none of the additional pharmacological treatments are going to be effective. If there is no heart to pump that blood, create that blood pressure, start perfusing the brain, start perfusing those major organs. Until we get that heart restarted, a lot of the other treatments are primarily going to be ineffective.

Q. Can administering certain medications affect the dilation of a patient's pupils?

A. Um, I can't speak to that as far as in a cardiac arrest situation. There are certain medications that will cause a dilation of a patient's pupils. There is certain chemicals that patients can be exposed to that are nonopioid-related that can cause a dilation of pupils.

But during the cardiac arrest situation, I don't

believe any medications we give will cause that sort of reaction.

Q. On May 21st, 2020, did you respond to a call for an unconscious male in his barracks?

A. Uh-huh. That's correct.

Q. Do you remember approximately what time you responded to that call?

A. I do not. I apologize.

Q. Do you remember whether it was day or night?

A. I believe it was daytime.

Q. Did you later learn the identity of the unconscious male, what his name was?

A. We generally do patient care reporting afterwards, so that would have been information I included in my patient care report. But to be honest, we run a lot of calls.

Q. Did you know that the unconscious male that you responded to the call for on May 21st, 2020, whether he was in the Marines or not?

A. I never actually was able to, say, ask anybody on scene, Was this an active-duty Marine? We talk about index of suspicion. We responded to the barracks on base. That's where active-duty Marines live, and that's where we responded to. Given his age, that's generally our assumption, but that's not something we verify as far as our taking of care priorities, whether or not he's active duty.

Q. Okay. When you arrived at the barracks, what was your role relative to the other medical personnel on scene?
A. Sure. So my -- it was my day to be the lead paramedic. I was assigned to -- to Medic 61, which is the ambulance that Marine Corps Miramar operates. And we responded with one of the fire engines that is standard for that to respond to as well.
Q. And on May 21st, 2020, what were your responsibilities as the lead paramedic?
A. So as the lead paramedic, it's generally a little bit less of a hands-on kind of role. You will get in there as much as required, but you're more there to kind of quarterback the incident, if you will.

So you're the one that assigns the patient respond -- the patient care responsibilities. You ensure that any tasks that you have assigned have been completed accurately. You are constantly reevaluating the status of the call and if any sort of therapies need to be changed.
Q. And when you arrived to to barracks, what did you see? Can you describe that for the jury.
A. Sure. I recall an unconscious male on the ground. There was law enforcement on scene. There was one law enforcement officer who, by happenstance, was also an emergency medical technician.

He was at the head of the patient, as we call it,

and he was already providing effective ventilations; so I actually kept him in that role. It's very tight quarters. It's really hard for us to work around. He was qualified. He was doing a fantastic job, so I kept him in that role, and then we assigned out tasks as per protocol.

Q. And when you arrived, was the patient in cardiac arrest?

A. Yes.

Q. Did you know why he was in cardiac arrest?

A. I did not.

Q. Did you ever determine why he was in cardiac arrest while you were on scene?

A. No, we did not.

Q. Did you ever try to determine it?

A. As part of our standard assessment, we always try and make that determination. But given our limited abilities and our limited diagnostic tools, it wasn't a hard-and-fast situation that we were able to make.

Q. What was your top priority when you entered the barracks and saw that the patient was in cardiac arrest?

A. Well, our top priority is always safety, to ensure that the first responders are going to be safe. Law enforcement was there, so that issue was addressed.

Beyond that, when I was speaking to organized assessments, we start with something really simple. It's

called ABCs. So airway, breathing, circulation. A law enforcement officer had already established an airway. He was helping out with that those respirations. So our next top priority is going to be compressions, so chest compressions.

Q. Did you ever hook the patient up to a heart monitor to monitor whether the patient's heart was beating?

A. Yes, ma'am.

Q. Now, did you ever see that the patient's heart was beating outside of any electricity or anything like that being administered?

A. No, it was not.

Q. And did you check to see if the patient was breathing on his own at any time, so independent of artificial ventilation being administered to him?

A. Absolutely.

Q. And from the time that you were there, was the patient ever breathing on his own?

A. No, he was not.

Q. Did you administer -- did you administer medication that's used to treat cardiac arrest?

A. Yes, we did. As part of our standard protocol, we administered the medication epinephrine.

Q. And can you explain, how does epinephrine work?

A. Epinephrine is actually, to put it simply, adrenaline.

It's actually another name for it. It causes stimulation of the cardiac muscle. Any sort of blood flow that does occur to that heart, any sort of electrical activity, the heart may start to generate on its own.

THE REPORTER: Wait. Wait.

MS. FORREST: I'm going to ask you to slow down.

THE COURT: You were doing terrific.

THE WITNESS: I know. I get a little technical and I get a little fast.

THE WITNESS: Correct. So any sort of electrical activity, the heart may begin to generate on its own. This medication will potentiate that. It will hopefully make it more effective.

Q. BY MS. FORREST: Now, regardless of the cause of someone entering cardiac arrest, in your experience, is cardiac arrest something that someone is likely to be able to be retrieved from, in your experience as a paramedic?

A. The data and statistics generally place that probability very low, generally the rate is 3 to 6 percent, is your survivability rate for a possible cardiac arrest.

Q. And is that 3- to 6-percent survivability rate once someone goes into cardiac arrest, is that percentage regardless of what puts them in cardiac arrest?

A. That is correct.

Q. And is that consistent with your 20 years' experience as

a paramedic?
A. It is.
Q. Now, was Narcan administered to the patient on May 21st, 2020?
A. Yes, it was.
Q. And what is Narcan?
A. I'm going to try to slow down because I'll go fast again.
Q. Thank you.
A. So Narcan is a medication that we administer to -- the common conception is that it's an antidote, that it works against any sort of opioid overdose. And in reality, what it is is it actually competes with the opiate that may be introduced into a patient's bloodstream. Everybody in this room has opioid receptor sites that are part of their general neurologic function. Your body naturally produces its own opiates.

Narcan more readily binds with those opiate receptor sites than an opiate will. So there has been occasions as well where we're administering Narcan, Narcan will wear off, that opiate will still be in that patient's bloodstream and they will become symptomatic again.
Q. Does Narcan save everyone that gets it, everyone who is suffering from an opioid overdose, if they are administered Narcan?

A. No, it does not.
Q. Have you had experiences where you've administered Narcan to someone as fast as you could, and you still weren't able to revive them out of a cardiac arrest?
A. Out of a cardiac arrest, I can't speak to that. I have been on opiate overdoses where we've given all the Narcan that we literally carried on that ambulance, and it still wasn't enough to compete with the amount of opiates that that person had taken.
Q. Now, you said that you administered Narcan in this case. Can -- can you explain to the jury whether that's part of the standard protocol in responding to a cardiac arrest?
A. No, it's not. It's not part of standard ACLS protocol. That was something that occurred later on in the incident. Generally, when we find a young, healthy individual and, you know, we're not really sure what the etiology of that cardiac arrest event may be; so we actually asked for that administration of Narcan as a physician variation.

So we have to get on the radio. We speak to a nurse that is at the hospital. There will be a physician standing right there with that nurse. We'll kind of tell them the situation, what we have done, that we want to try everything. And that's how the Narcan order came about.

Q. So is it the case that you can't just decide, hey, from what I see, I need to administer Narcan? You actually have

to contact a hospital and get authorization to administer Narcan?

A. Not in all situations. Narcan administration is part of a lot of our standing-order protocols. In the instance where we are working a cardiac arrest, it's not part of that standard algorithm. So if we do want to make that administration, we have to ask for a variation from a physician to make a change to that protocol.

Q. So to make sure I understand. In cases of cardiac arrest, you have a standard protocol, and Narcan is not part of that standard protocol. That requires getting special authorization from a physician; is that right?

A. That is correct.

Q. And how does the physician provide this special authorization while you're on scene, in the barracks, for example, in this case?

A. We communicate via radio system.

Q. Now, do you personally communicate via radio system?

A. Very frequently, that's my primary responsibility. On this particular incident, one of our emergency medical chiefs had shown up to the response, and that was a task that I delegated out to him.

Q. And what is that chief's name?

A. Matt Rios.

Q. So what did you have him do?

A. So he's also an extremely experienced paramedic. He was one of our primary EM Chiefs. And basically give him a turnover as far as what we had done, what protocols we had followed, what our current situation was and anything else that we had to try.

So he would get on the radio. He would contact our base hospital. And then he would give that same full report to a nurse, who's on the other end of that radio, and given the circumstances, she'll generally request a physician to come into the radio room, and that whole sequence of events occurs again. And then treatment decisions are made after that.

Q. And so after you obtained authorization to provide Narcan, can you explain what exactly you did?

A. Yeah, it's a fairly straightforward procedure. We already had access to the patient's vasculature. It's as simple as taking the medication out of the box and hooking it up to the line, the access line, and simply pushing the medication into the patient's bloodstream.

Q. And when you administered -- did you yourself administer the Narcan?

A. I believe I did. I was -- like I say, it's been a couple of years. I believe I was the one that pushed the Narcan.

Q. Okay. Do you recall what happened after the Narcan was

administered?
A. Nothing.
Q. Did it have any effect on the patient's breathing? Did it suddenly start his heart?
A. No, it did not.
Q. Did it have any effect on his ability to breathe on his own? Did it suddenly make him start breathing?
A. No, it did not.
Q. You used the term earlier "index of suspicion" with respect to a determining or attempting to determine the cause of someone falling into cardiac arrest.

Can you explain what you mean by index of suspicion.
A. Sure. Depending on the response you're going to, so, for example, if we go to a patient's house that is 95 years old and has a long list of preexisting medical conditions, problems with their heart, problems with their lungs, our index of suspicion is it's probably related to something like that.

If we go to, you know, a middle-aged male, those indexes may change. We may be concerned about, is it something that occurred secondary to high blood pressure or, you know, pulmonary embolisms, traumatic events. And it's very case dependent.

So depending on the individual, the circumstances, what we see on scene, that will kind of influence our index

of suspicion, as far as what may have caused that incident to occur.

Q. And so specifically focusing on opioids and your experience seeing opioid -- or treating people who have suffered from opioid overdoses, what are some of the factors that contribute to your index of suspicion, whether it's high or low?

A. A lot of that -- excuse me. A lot of that has to do with what we can see on scene. Do we see paraphernalia? Are there bystanders on the scene that witnessed the event occur? Does our suspect reveal pinpoint pupils? Does it reveal -- I mean, I've been on cases where we'll go on heroin overdoses, which is a similar sort of drug, and you'll still find the needle hanging out of the arm, so your index of suspicion is quite high for that. But as far as, you know, making solid determinations on everybody, it's pretty tough to do.

Q. Did you have a high index -- or I should rephrase.

What was your index of suspicion as to whether the patient you treated on May 21st, 2020, whether it was an opioid overdose that caused him to enter cardiac arrest?

A. At that time, it really wasn't high on my suspicion list.

Q. Why not?

A. In my experience, prior to working for MCS Miramar, I worked downtown San Diego, and we frequently encountered

people that were utilizing these sort of substances for desired effect. It wasn't something that we saw very often in MCS Miramar. It was a pretty rare event for us to actually push Narcan on a patient.

MS. FORREST: One moment, Your Honor.

THE COURT: Yes.

MS. FORREST: No further questions, Your Honor.

THE COURT: Okay. Thank you.

Mr. Cortez, cross-examination.

MR. CORTEZ: Yes.

CROSS-EXAMINATION

BY MR. CORTEZ:

Q. Is it Officer Naylor or Mr. Naylor?

A. Mr. Naylor is fine, sir.

Q. Mr. Naylor --

A. Yes.

Q. -- let me pick up where my colleague left off.

She was going over your index of suspicion as to what may be the medical urgency you have. You did not have a high degree of suspicion that this was an overdose of opioid; right?

A. At that time, I did not, no.

Q. And you just testified just now that you had a lot of experience with overdose cases, especially opioids; right?

A. That is correct.

Q. Downtown?
A. Yes.
Q. And you added that sometimes you even see someone with a needle sticking out of their arm, and that's pretty easy to determine what may be going on?
A. Absolutely.
Q. But curiously, you also mentioned as part of your determination about what is happening here, this patient, you look at their pupils; right?
A. That is correct.
Q. And the pupils in this case were dilated; right?
A. That is correct.
Q. Now, earlier you testified, also on direct examination, that you gave Mr. Reed medication to try to get that pump going; correct?
A. That is correct.
Q. And he was being oxygenated already by Captain Balderson, correct, by the time you got there?
A. I don't -- I don't know if he actually had oxygen with him.
Q. You testified on direct that when you walked in, you determined it was a cardiac arrest, and that one of the things to do immediately is to oxygenate the person; right?
A. There is a difference between oxygenation and ventilation. So ventilation --

Q. I'm sorry. I'm sorry.
A. Ventilation is actually providing that positive pressure --
Q. Correct.
A. -- to simulate respiration.
Q. Correct.
A. Oxygenation is when you actually apply oxygen to that device.
Q. Correct. And in this instance, when you came in, there was an officer already intubating the patient; right?
A. That is not correct.
Q. Did you see Captain Balderson? You said you saw an officer that was doing a pretty good job already?
A. Correct. But he's not qualified to provide intubation.
Q. I got it.

What was he doing a pretty good job of?

A. On providing basic ventilations, according to the American Heart Association standard protocol for a basic provider.
Q. I got it.

For all of us who are not trained, what is ventilation?

A. Ventilation is basically simulating your natural response to breathe, but you're doing it mechanically for

them.
Q. When you say "mechanically," so that we all can understand, you're pushing down on the chest area?
A. That is different than ventilation. When you're pushing down on the chest, you're providing compressions.
Q. I see.
So how do you provide ventilation to a patient?
A. So you provide ventilation to a patient, from a basic standpoint, we'll generally place, it's a plastic device that goes into their mouth.
Q. I'm sorry. You said a what device?
A. A plastic device.
Q. Plastic device on the face?
A. It goes inside the mouth. It's called an oropharyngeal airway.
Q. Oral what?
A. Oropharyngeal airway.
Q. And "pharyngeal" because it's in the pharynx?
A. That's correct.
Q. Does it have a bag attached to that?
A. It does not.
Q. How does it provide ventilation?
A. The device does not. What the device does is help maintain the airway in an open position, so that when we use that device, that bag-valve mask, that when we go to provide

that ventilation, it's not occluded by the patient's tongue.

Q. I see. And you were watching someone ventilating that patient; right?

A. Absolutely.

Q. And at that time, there was no bag attached to that thing he had in his mouth?

A. When the first responders arrived on scene, when the paramedics arrived on scene?

Q. Yes.

A. We absolutely utilized that device.

Q. All right. But when you arrived there?

A. I don't recall.

Q. Okay. At any rate, you testified on direct that you felt comfortable because who was giving ventilation was doing a pretty good job; right?

A. Given the proper equipment, he was doing a good job.

Q. Do you know who Captain Balderson is?

A. Do I know who he is?

Q. Yes.

A. Just from in a professional nature, responding on incidents with him, and it was relayed to me on previous incidents that he was an emergency medical technician.

Q. Did he arrive there before you?

A. He did.

Q. So by the time you got there, Captain Balderson was already attending to the urgency; right?
A. That is correct.
Q. Thank you.
Now, you were asked whether the administration of Narcan would do any good when there is no palpitation of the heart; right?
A. I believe so.
Q. And you said, well, no, because you've got to have circulation first in the body for those medications to get in; right?
A. That is correct.
Q. Okay. And you had administered some medications, one of them was epinephrine; right?
A. That is correct.
Q. Because you were focused on getting that heart going, that pump going, that circulation going; right?
A. That's the primary objective, yes.
Q. And as you're in the middle of trying to work on this patient, you're also getting feedback and back and forth with the hospital; right?
A. Not during the initial stages of the event. Generally we make base hospital contact when we've exhausted all of our standing-order protocols and we're looking for additional treatment methodologies.

Q. And when you got to that point, the chief basically -- Chief Rios basically called the hospital, right, to get guidance?

A. That is correct.

Q. Okay. And so when the chief calls the hospital, you're telling the hospital people, the nurse that you mentioned, the doctor that may be consulted, everything that you're encountering with that patient at that time; right?

A. That is correct.

Q. Because that detailed information regarding the patient's health is critical to the doctor who is not on scene; correct?

A. That is correct.

Q. And knowing all of the factors that you relayed to this doctor at the hospital, even though you said that sometimes Narcan doesn't do any good after you get the heart pump going, the doctor told you to try Narcan anyway; right?

A. I believe -- I wasn't on the radio to exactly know the exchange of verbiage that occurred. However, when Chief Rios came back into the room, the determination was already made to provide that Narcan.

Q. Exactly. Because the doctors off-site believed it might help; right?

A. We wouldn't have administered it if we didn't believe it would have.

Q. Exactly.

MR. CORTEZ: That's all I have. Thank you.

THE COURT: Thank you.

Redirect Examination?

MS. FORREST: Yes.

REDIRECT EXAMINATION

BY MS. FORREST:

Q. Who asked to administer Narcan to the patient on May 21st, 2020?

MR. CORTEZ: Asked and answered, Your Honor.

THE COURT: Overruled.

THE WITNESS: Can you repeat the question again?

Q. BY MS. FORREST: Who asked for permission or authorization to administer Narcan to the patient on May 21st, 2020?

A. I believe that would have been Chief Rios.

Q. And was Chief Rios a member of your team?

A. Yes, ma'am.

Q. Is it fair to say that your team requested permission to administer Narcan to the patient?

A. Yes, ma'am.

MS. FORREST: Nothing further, Your Honor.

THE COURT: Okay. Thank you, sir. You may step down.

THE WITNESS: Thank you very much.

THE COURT: Government's next witness, please.

MR. ROTH: Thank you.

MS. FORREST: Yes, Your Honor. The United States calls Dr. Platt.

THE CLERK: Thank you. Please raise your right hand.

BRYAN PLATT, MD, a government witness, having been duly sworn, testified as follows:

THE WITNESS: I do.

THE CLERK: Thank you. If you can please state your name and spell it for the record.

THE WITNESS: Bryan Platt. Bryan, B-r-y-a-n, Platt, P-l-a-t-t.

MS. FORREST: May I proceed, Your Honor?

THE COURT: Yes, you may.

MS. FORREST: Thank you.

DIRECT EXAMINATION

BY MS. FORREST:

Q. Good afternoon, Doctor.

A. Good afternoon.

Q. Now, Doctor, I'll tell you what I tell all witnesses. Please try to speak up, speak into the microphone; we have someone taking down everything you're saying. And please try to speak slowly, which I know can be tough to do sometimes.

What do you do for work?

A. Currently, I'm one of the deputy medical examiners at the Armed Forces Medical Examiner System at Dover Air Force Base in Dover, Delaware.

Q. Can you explain for the jury what your responsibilities are as the deputy medical examiner.

A. As one of several deputy medical examiners, I am responsible for performing forensic pathology investigations, including autopsies, on cases that come under our jurisdiction.

Q. You mentioned forensic pathology investigations. Besides autopsies, what do such investigations include?

A. Well, the -- the phrase "forensic pathology investigation" is basically the sum total of everything we're doing there. It's investigation at the scene, the toxicology, the histology, the -- you know, the identification process; that is the phrase that we utilize in our office to encompass the whole process.

Q. And so that includes autopsies and toxicology results taken of an individual?

A. Correct.

Q. And toxicology -- can you explain what a toxicology report or result -- report is or results are for the jury.

A. Well, for toxicology, in our office, we utilize specimens derived during the autopsy examination. We will collect -- in our office, we collect blood. We collect

urine, bile from the gallbladder, the vitreous fluid from the eye and gastric contents. We will also additionally utilize as something of a backup, as necessary, we utilize solid organs, most commonly the liver, the spleen or the kidney.

Q. Now, what is your background that you have that qualified you to become a deputy medical examiner in your current position?

A. All the way back?

Q. Well, you don't have to go back to junior high, but...

A. So I have an undergrad in chemistry from Albright College in Reading, Pennsylvania in 2001. Went to medical school at Temple University School of Medical, now the Katz School of Medicine. Graduated there in 2007.

At that point, I transitioned to Naval Hospital Pensicola. I did an internship year in family practice from 2007 to June of 2008. I stayed in Pensicola for approximately another eight months, going through the Navy's flight surgeon billet -- or training there. After that, I was assigned to a pair of Marine CH-53 helicopter squadrons at Marine Corps Air Station, New River, North Carolina. That's the air station associated with Camp Lejeune. I was there from April '09 until August of 2011.

At that point, I was -- I started my residency in anatomic and clinical pathology at Walter Reed National Military Medical Central in Bethesda. So that was from

September of 2011 until August of 2015.

I started my forensic pathology fellowship at the Virginia Office of the Chief Medical Examiner, at their Tidewater Regional Office in Norfolk in November of 2015 through October of 2016. And then at that point, I trans -- I moved to the Armed Forces Medical Examiner system and have been there ever since November of 2016.

Q. And throughout your career in conducting autopsies, if you had to put a number on it, approximately how many autopsies have you conducted?

A. Fellowship, 201. For the -- for AFMES, I've done I believe an additional 135 at this point.

Q. And this might be a weird question, but how do you have such a specific number of the number of autopsies you've performed?

A. I keep Excel spreadsheets of all the details on them. I do that to track all the pieces as they come in. All the toxicology, where it is on the review process. So it's something that I developed in fellowship and I just kind of stuck with it.

Q. Now, did you perform an autopsy of a male by the name of Christian Reed in connection with this case?

A. I did.

Q. Now, before we get into the details of that autopsy, can you kind of orient the jury, sort of the information that you

need and what you do before you begin your autopsy?
A. Okay. So before we begin our autopsy, typically -- you know, our office is different in that we're the only federal entity out there. So it takes a little more time to establish whether it's our appropriate jurisdiction. The phrase we use is "exclusive federal jurisdiction." So most commonly, you know, deaths on base, deaths on ships, deaths overseas, things like that.

So initially we are presented with a death. We confirm that it's our jurisdiction. And then we are given kind of the basic synopsis of what was found. You know, found unresponsive in their barracks room, found unresponsive outside, you know, found hanging, found with a gunshot wound to the head. Usually it's that kind of brief synopsis that we are given initially prior to beginning the case.
Q. Now, is the synopsis usually pretty general, or is it very specific? Does it vary?
A. Well, it's very concise is the best word I would use. So basically, you know, just basically what is the minimum that is known at the time the death is presented to us to make a determination, A, is it our jurisdiction; and, B, is it a forensic case or not.
Q. And what do you mean by "forensic case"?
A. When we talk about medical examiner work and forensic pathology, we're talking about the investigation of sudden,

violent, unnatural, unexpected death; so we need to hear enough circumstances to fit those categories.

You know, so a lot of times it's from the moment -- from the moment the remains are discovered, maybe -- like I said, a hanging, gunshot wound to the head, yes, it's obvious that it's a forensic case, but sometimes if it's someone who has died in a hospital, and we have to understand the circumstances of why they were admitted to the hospital first before we decide whether it's our case or not, whether it would fit for -- you know, fit into that definition.

Q. Do you recall when you performed the autopsy of Christian Reed in connection with this case? Do you remember the date of your autopsy?

A. The date?

Q. Yes. Do you recall --

A. Can I refer to my report?

Q. Of course. You can turn to -- it looks like you may have your report with you.

A. Uh-huh.

So the date of death we were made aware of, it was May 21st of 2020.

Q. And what was the date that you performed the autopsy?

A. May 28th of 2020.

Q. Now, is that a typical time frame before you start an autopsy?

A. At that time, you know, in response to travel restrictions placed throughout the Department of Defense, we could not operate as we normally would. For deaths that occurred in the United States, typically what we will do is we'll travel to the case, because it is much more difficult to move someone, you know, across state lines before they are embalmed and ready to be interred.

So because of that -- so at that time what we were doing in response to the travel restrictions is we were moving cases to where we were located. Dover Air Force Base. We have some regional medical examiners in Texas. We have a regional medical examiner in Washington State. So the time it takes to coordinate that contract, that -- obtain permissions in the State of California, we had to -- there were forms that we had to fill out specifically to get the remains from California to Delaware. So we -- that seven-day time frame was not uncommon during that period.

Q. Now, do you recall the date that you signed your autopsy report?

A. The date I signed it was August 7th of 2020.

Q. And what is the reason that you perform the autopsy on, I believe it was May 28th of 2020, and you signed the report in August of that same year?

A. Well, when -- the standard for completing our autopsy reports is 90 percent of our case is finished within 90 days.

So without doing the math, I mean, I know it's within standard at that point.

But we're also -- we're waiting on numerous things to come in, toxicology results. If I have to -- if I need to engage a neuropathology expert to look at the brain with me, that takes some time; a cardiovascular cardiologist expert to look at the heart with me, that adds some time, in addition to any other special studies that are required. Plus, also, you know, other -- you know, life also gets in the way as well sometimes.

Q. Do you always consult or -- I believe the word you used is "engage." Do you always engage these specialists as part of your autopsies, or is it under special circumstances?

A. Yes, it's under special circumstances. It's not a routine practice. If it's a -- you know, if there are circumstances that you -- you know, like I said, if you do not have a good answer or a good diagnosis at the time, there is nothing wrong with engaging an expert at that point; but it is not done routinely.

Q. Now, what were you informed as far as the circumstances of Mr. Reed's death before you began your autopsy?

A. So initially we understood he was found unresponsive in his barracks room during a welfare check. And kind of the use of that term is, okay, well, did he not show up to a formation? Did he not show up to an appointment, what have

you. In clarifying that, we also learned that he had been recently admitted at the Naval Medical Center in San Diego approximately a month prior to death.

So, you know, those two pieces of information, initially before he arrived at Dover, the question was, okay, are we looking at something totally separate or are we looking at something that may be related to why he was admitted a month earlier.

Q. And was that one of your first considerations, whether his hospital admission a month prior to his death in any way related to his death?

A. Yeah. That was kind of the first big branch point in our -- kind of in my thinking in my mind. Okay. What do I have? What question do I have to answer? Okay. Is this part of the hospitalization or is it something else?

Q. So how did you go about answering that question for yourself?

A. Well, I mean, it's also going through the autopsy and looking specifically for that. It's more of just -- you know, the phrase I use is if you don't think about it, you're not going to look for it. So it's just being aware of that while I'm doing the same process for an autopsy I do with my other cases.

Q. Is it standard for you to review an individual's medical history, either before or after you conduct your autopsy?

A. Either/or. I've done both, you know, depending on the circumstances. If I have questions or something doesn't make sense to me, I'll look in depth.

The other instance when we will go through the medical records is like I talked about earlier, in determining whether a case just is a medical examiner case or not. So that -- so -- so there is a variability as far as how deeply we go through the medical records before versus after the autopsy.

Q. And so, for example, such as this case, where you're informed that an individual was hospitalized a month before their death, did you take the steps to review Mr. Reed's medical records, to either rule out or consider whether that hospitalization contributed to his death?

A. I did. The big thing I was looking for there is, well, what was his admission diagnosis? What was his discharge diagnosis? What did he show up with, what was diagnosed during the hospital course and what was he discharged with?

Q. Let's start with admission diagnosis. Why did you look to see what the admission diagnosis was?

A. Well, honestly, just to kind of see, okay, what -- you know, literally, what parts of the body do I got to spend some scrutiny on. The initial story was altered mental status. Okay. I've got to pay attention to the brain, probably something going on there. They talked about arm

weakness or something like that; I'm going to look for injuries. Is there something to explain that as well?

And then along with that, you know, specifically talking about mental status, okay, what did the radiology of the brain show at the time? And so that's -- so those were kind of the questions I had going into that autopsy.

Q. And why did you look at the -- why did you in this case and why do you generally look at the diagnosis someone has when they are released from a hospital?

A. Mostly, just to see how things may have progressed. You know, did they resolve? Did they -- did it resolve? Did the admission diagnosis change, you know, to a final discharge diagnoses? Did they find anything additional? That's -- basically it's comparing those two points in time.

Q. If you look in the binder -- there is a binder near you. There should be a disc titled Exhibit 5. It's a CD. Can you let me know when you see that?

A. I see it.

Q. Now, did you have an opportunity to review the contents of this disc?

A. I did.

Q. And what is on this disc?

A. What it is is a -- it's a PDF file that -- the medical records system we use, we call it ALTA, in the military, that it creates a medical record, mostly outpatient, but also some

inpatient stuff, particularly with labs and studies. And it organizes it into a nice PDF file with different sections and it organizes by page number. So a lot of times that is -- when we engage -- particularly medical, other military medical hospitals, and we engage their patient records, that's the format we get.

Q. And did you have an opportunity to review Mr. Reed's medical records contained on this disc?

A. I did.

Q. Did you have an opportunity to review the toxicology results of tests that were performed on his body at the time of death?

A. At the time of death?

Q. Or after death, I should say.

A. So you're talking about my studies?

Q. Yes.

A. Yes. Yes.

Q. And is there any information, other than what I've mentioned that we've already discussed, that you considered in coming to your conclusions regarding the cause of death in this case?

A. Yes. So just to backtrack a little bit. When -- going through his autopsy, I didn't find any significant injuries or anything that would be considered a cause of death. Particular to the brain in this case, there was concerns that

there were some findings in the brain that were possibly contributing to the arm weakness that they had talked about.

At the -- I had seen -- I went through and I saved the brain at that time to review with my neuropathologist. And the brain is a unique organ. In order to do that, in order to do your best exam with it, you have to preserve it first versus seeing -- you know, examining it right then and there at the time of autopsy.

So while that was occurring, and then the damage to the brain, going throughout the rest of the body, nothing else that I could identify as a cause of death. I received my toxicology results then, and it was -- in those results, I was able to settle on a cause of death in this case.

Q. And Dr. Platt, what did you determine to be the cause of Christian Reed's death?

A. So I certified this death as an acute fentanyl intoxication.

Q. Can you explain to the jury what that is.

A. Basically, in everyday language, it was an overdose. He had a fatal level of fentanyl concentration in his blood.

Q. Do you recall the level of fentanyl that Mr. Reed had in his blood?

A. It was 99 nanograms per milliliter.

Q. Can you explain for the jury what that quantity is with respect to an amount of fentanyl that is necessary to end

someone's life.

A. Yeah. So -- well, it's -- in toxicology in general, it's not just enough to say that when you're talking about something is a fatal level, there is one number and everything above it, everyone is dead and every number below it, they are not going to be. That's not the case.

With fentanyl, it does have proper -- you know, we use it in the hospital all the time. We use it primarily for pain relief, analgesia. At that range, we're talking about low single digits, one, two nanograms per milliliter.

Fentanyl also has use in the Intensive Care Unit, in the ICU, when, you know, someone is intubated and for more anesthesia purposes. And that range is in the low single digits. 10, 12, 14, you know, up to 20 nanograms per mill. Because you can give those higher doses because you're already supporting the person's breathing. Once you get above that low, single-digit level, you start to affect the, what we say, respiratory depression there.

So 99 nanograms per mill is well within the accepted range in the medical examiner community of, yes, this is consistent with a fatal overdose.

Q. When you reviewed Mr. Reed's medical records and performed your autopsy, did you specifically look for any evidence that Mr. Reed had had a stroke a month prior to dying?

A. So part of the -- where that stroke comes up is with the imaging, he was in the hospital a month before he died, there were some findings that suggested maybe there was an area of an infarct or a stroke, particularly in the right motor cortex, that could be causing problems with his left arm.

After -- after I received my toxicology result, I was very satisfied this was going to be a cause of death, so -- but then for completeness' sake, knowing this history, I went ahead and examined the brain myself, with the caveat of, okay, if I see anything suspicious, I can still engage my neuropathologist.

So I went through and sectioned the brain. I didn't see any lesion that corresponded to what was described on the radiology. Everything looked essentially symmetrical, and that's the first thing I look for, does the right side look like the left side?

And then I took several sections of the brain to look at under the microscope, because one of the other issues identified in the imaging was a concern for basically a vasculitis, an inflammation around the vessels, that was leading to this altered mental status, this encephalitis picture that clinically they were seeing. And I didn't appreciate any inflammation on the various slides that I looked at.

Now, that's -- you know, looking through his medical

records, he had received a course of steroids. These would basically calm down the immune system, so it's not unreasonable that, okay, you know, the reason I'm not seeing any of this is he was treated adequately and the issue had resolved.

Q. Did you find any evidence in Mr. Reed's brain or in any other part of his body that he had had a stroke a month before he died?

A. No.

Q. And you mentioned vasculitis. Do you believe that it's possible Mr. Reed did, in fact, have vasculitis a month before he died?

A. It's possible. They described altered mental status clinically as part of his initial presentation. So there were radiology findings suggestive of it. But there was -- like I said, he was discharged. There was an interval where, by all accounts, he had returned to basically baseline functioning. So for me to not find anything, you know, it is not proof of, well, that was an incorrect diagnosis coming in the door. No, that's not what I'm saying.

Q. So given that it may have been the case that Mr. Reed had vasculitis a month before he died, why don't you think vasculitis had anything to do with the cause of his death?

A. Well, I mean, it's -- I'm not sure I'm following your question.

Q. That's my fault. Let me rephrase.

Given that you determined that it was possible that Mr. Reed had suffered from vasculitis a month before he died, why did you not conclude that that was any sort of contributing factor to Mr. Reed's death, his cause of death?

A. Well, No. 1, it wasn't present at the time of my autopsy; so that's probably the biggest factor.

Q. If Mr. Reed had died from vasculitis, would you expect to see evidence of that when you performed your autopsy?

A. If it was a vasculitis encephalitis picture, yeah, I need to see it if I'm going to call it that, yeah.

Q. Is there anywhere, other than the brain, that you can find evidence of a stroke in someone's body?

A. Well, a stroke, yeah, the long term for a stroke is a cerebral vascular accident; so by definition, yeah, it's the brain or, you know, the mid-brain, basically the central nervous system.

Q. And what do you expect to see when you look at someone's brain and they have died from a stroke?

A. Well, if they had died acutely, you know, like within hours, days, you know, from that, it depends on how it was -- what caused it. Whether it was a hemorrhagic stroke, you know, blood vessel bursting, there is a large bleed. Well, that's going to be obvious.

The other thing -- the other big category of strokes or cerebral vascular accidents you see are more of the embolic strokes. These are ones where he had the blood clot in the leg, he had a hole in his heart, or they had atrial fibrillation, blood clots in the heart, and they got spread up into the brain. That, what you see there, with that blood flow being cut off, you start to see basically the -- that territory of the brain breaking down, starting to become necrotic. So it -- it follows the distribution of the vessels, as far as what regions you see.

And there are a number of changes you see, depending on at what time, you know, how soon or long after the stroke has occurred, as far as what you can see both initially at autopsy and then under a microscope.

Q. Focusing on fentanyl causing someone's death. You said earlier that there are medical uses of fentanyl; is that right?

A. Correct.

Q. Now, when you look at the results of an autopsy -- or excuse me, your conclusions, when you determine that someone has died as a result of fentanyl, is there anything about the fentanyl that allows you to determine whether it was from a medical source or it was an illicit source?

A. So the easiest way, the most common way we look at it is if there is anything basically that shows up in the

toxicology with it. If it's a purely medical source, we're going to see fentanyl and, you know, some of the metabolites, norfentanyl, 4APP -- 4APPP. That's the abbreviated version.

Sometimes we will see compounds that we know have no human use, you know, things that are used in the veterinary space, so, you know, colloquially, they are referred to as cutting agents.

Q. And so do -- does the presence of cutting agents tell you that the fentanyl in the person's system did not come from a medical professional?

A. Yes. That signals to us that most likely, if we see a compound that we know has no human use, it is likely a cutting agent.

Q. And did you find the presence of any cutting agents in the fentanyl -- or excuse me, in Mr. Reed's body in addition to the fentanyl levels?

A. Yeah. The agent that we found on our toxicology screen, there was one agent, and that's a metabolite. Referring here from the toxicology report, 4-Methylaminoantipyrine, and then its metabolite 4-Aminoantipyrine. Those compounds, in and of themselves, have some veterinary use. They have no use in humans. So that could be a source.

The other possibility of the source for specifically the 4-Methylaminoantipyrine, it is the metabolite of a very old, you know, old pain reliever, old fever reducer that used

to be on the market in the U.S. about 50 years ago. The generic name is Matamizole. You can still find that drug around the world, you know, Central America, South America, China, Russia. There is still some countries in Europe that have it. So that could be also another possible source, but, again, it's not in the market in the U.S.

Q. When examining someone's body, so performing an autopsy, are there any sort of classic signs that you see in performing autopsies that are consistent with an opioid overdose, or any sort of drug overdose?

A. The two big things that we look for that we note, as far as an opioid overdose -- and this is just in general -- is the lungs tend to be -- they tend to be more full of fluids, pulmonary edema.

And that's just the -- from the -- you know, as the opioid is a narcotic, is it slows down your respiratory drive. You don't respond appropriately to the buildup of carbon dioxide because you're not adequately exhaling it out. Your body is slowly becoming more hypoxic, more deprived of oxygen, so in that setting, you start to accumulate more fluid in the lungs.

The other thing we see, and there are some studies that kind of tease this out with various opioids, between heroin and fentanyl and some of the other pills, is there will be some urinary retention. But that, you know, is not

always the case. It doesn't have to be there. But it's -- it's things that kind of point you in that direction prior to receiving your toxicology results.

Q. Given Mr. Reed's height, weight, his age and size generally, what would you expect -- what would have expected his lungs to weigh when you performed your autopsy?

A. For his height and weight, young adult male, the 300- to 400-gram range is what I would anticipate.

Q. Did you weigh Mr. Reed's lungs?

A. I did.

Q. Do you recall the weight of both of his lungs, the separate weight of each lung?

A. So from my report, the right was 860 grams; the left was 900.

Q. So was the weight of Mr. Reed's lungs individually consistent with your expectation that there is pulmonary edema or fluid retention in the lungs of someone who dies of a drug overdose?

A. Yes. The weight, and then also when I looked at the lungs under the microscope, you can see the fluid in there as well so...

Q. You mentioned some, I believe it was steroids or prescription medication that Mr. Reed was taking. Did you review his medical records for all of the medication that he had been prescribed?

A. Well, I can't quote all the medications he had been prescribed, but, you know, I do recall that as part of his treatment course, he did receive steroids for that.

Q. And looking at the medication that was in his system, in addition to any other substances as far as the toxicology report, did any medication that Mr. Reed took, in your professional opinion, contribute in any way to his cause of death?

A. As far as the additional findings on the toxicology screen, no, it was just the fentanyl, the fentanyl and the fentanyl metabolites.

MS. FORREST: One moment, Your Honor.

THE COURT: Yes.

MS. FORREST: No further questions, Your Honor.

THE COURT: Okay. Thank you.

Mr. Cortez, Cross-examination.

MR. CORTEZ: Yes. Thank you.

CROSS-EXAMINATION

BY MR. CORTEZ:

Q. Dr. Platt, let me begin a little bit where she left off.

You mentioned metabolites of fentanyl; correct?

A. Correct.

Q. And you know what metabolites of fentanyl are in a body; correct?

A. I -- yes.

Q. In fact, you listed them in your report; right?
A. Correct.
Q. Do you have your report in front of you, sir?
A. I do.
Q. I'm going to refer you to Page 6. Tell me when you're there.
A. Got it.
MR. CORTEZ: If I can refer you to Roman Numeral Section IV.
THE COURT: Do we have a copy of this exhibit that's been marked for identification?
MR. CORTEZ: I do. I can introduce it.
THE COURT: Just so the record is clear, what he's referring to and so we have a copy of what he's referring to.
MS. FORREST: Yes, Your Honor. We do have a copy. It's Exhibit 4. I believe Dr. Platt has his own copy of the exhibit, but it's the same document, Your Honor.
MR. CORTEZ: Exhibit 4.
THE COURT: Okay. Thank you.
Q. BY MR. CORTEZ: Exhibit 4 in front of you, that's the autopsy report you signed; correct?
A. Correct.
Q. And you signed it -- if I may have just a second -- on August 7th, 2020; correct?

A. Correct.
Q. At 9:56 in the morning?
A. Yes, sir. That's the time stamp, yes.
Q. That's the military stamp?
A. Yes.
Q. And your report consists of seven pages; correct?
A. Correct.
Q. And this report is the autopsy report you prepared for this case; correct?
A. Yes.

MR. CORTEZ: At this point, I offer it as -- is it in for the government?

MS. FORREST: The United States did not introduce the report, Your Honor.

MR. CORTEZ: I offer it -- I'm sorry. Go ahead.

I offer it as Defendant's 1.

THE COURT: The only marked copy that I have is Government's 4. Do you want to introduce Government's 4?

MR. CORTEZ: I have it in front of me, except it has a government sticker and mine now.

THE COURT: Okay.

MR. CORTEZ: So should I identify it as Defendant's 1?

THE COURT: Sure. That's fine. You can identify it however you see fit. I just don't want two exhibit stickers

on a single exhibit, please.

MR. ROTH: We're happy to refer to it as Defendant's Exhibit 1 going forward, Your Honor.

THE COURT: Okay. Is there any objection to Exhibit 1 being received in evidence?

MS. FORREST: No, Your Honor.

THE COURT: Okay. Defense Exhibit 1 is received in evidence.

(Defendant's Exhibit 1 received in evidence.)

MR. CORTEZ: Thank you.

Q. Let me take you now to the page I was asking about, and that would be Page 6, Roman Numeral IV.

Doctor, are you there?

A. Uh-huh.

Q. This section is entitled "Toxicology and Special Studies"; correct?

A. Correct.

Q. And you list first two categories. A, it's volatiles; correct?

A. Correct.

Q. And, B, drugs; correct?

A. Correct.

Q. And you have ten drugs listed under that section; correct?

A. Correct.

Q. The first one is fentanyl?
A. Yes.
Q. The 99 nanograms per liter that you mentioned on direct; correct?
A. Yes.
Q. The second one is norfentanyl; correct?
A. Correct.
Q. And please tell the jury what norfentanyl is.
A. Norfentanyl is most commonly a metabolite of fentanyl.
Q. And, Doctor, for those of us who are untrained. What is a metabolite?
A. Basically a metabolite, it just refers to a change in the apparent drug that the body does, whether it goes through the stomach or it goes through the liver or it goes through the kidney, the body will do things to the drug to make it more soluble or also to -- you know, as far as to excrete it.

So if you're talking -- so from fentanyl to norfentanyl, there is some chemical change somewhere, it's usually very slight, but it -- but it changes -- it changes the overall structure. It changes, and, thus, there is a name change.

Q. And so in layperson's terms, if I may, norfentanyl is how the body breaks down fentanyl; is that fair to say?
A. Yes. Yes.
Q. Thank you.

Item 3 is 4-Amino-1 -- can you help me pronounce that big word?

A. 4-Amino-1-phenethylpiperidine.

Q. Also known as 4-ANPP; correct?

A. Yes.

Q. What is that?

A. So that is, again, another metabolite of fentanyl. Why it uses the 4-ANPP name, I don't know. But the only thing I would point out, that name, is it refers to the piperidine at the basic structure of fentanyl. There is a six-member ring where if you're -- if you remember any chemistry, instead of six carbons, there is five carbons and a nitrogen, so it just refers to the basic structure there.

Q. Without getting into the chemistry, so this chemical or molecule is a metabolite of fentanyl; correct?

A. Yeah. So it either came from fentanyl or the body further metabolized norfentanyl to get to that.

Q. Correct.

Would it come from another opiate?

A. No.

Q. Are you sure?

A. Yes.

Q. Absolutely sure?

A. Unless you have other opioids.

Q. I don't, sir.

A. No. Because the structure of fentanyl is very unique compared to things such as heroin or morphine. Morphine, you're talking about a four-ring structure altogether. These are -- you know, these are three-ring structures that are on -- kind of stretched out on a chain, basically is the simplest way to describe it.

But, no, these are not going to go back and forth from something like heroin or, you know, some oxycodone or OxyContin or anything like that.

Q. Oxycodone you said?

A. Yeah. It's not going to be crossover back and forth. These are very, very different structures.

Q. Oxycodone has another name, Percocet; correct?

A. I believe that is, yeah, oxycodone/Percocet, but, yeah, that's possible.

Q. And then you list -- go ahead. I'm sorry.

A. No, I believe you're right.

Q. Okay. Thank you, Doctor.

Items 4 and 5 are also metabolites of fentanyl; correct?

A. No.

Q. Huh?

A. No.

Q. What are they?

A. They are -- you know, like I was explaining, these

are -- these are all -- these are pyrene molecules that have --

Q. Wait. I'm sorry to interrupt you. But we don't -- what is the piperidine part?

A. Well, it's simplest to say, no, these are not metabolites of fentanyl.

Q. Okay.

A. These compounds, like I said, have had -- they have no human use --

Q. Okay.

A. -- as described right then and there.

Q. Thank you.

Item 6, 7, 8, 9 and 10 are all the medications Mr. Reed was on; correct?

A. Correct.

Q. Gabapentin, what is that for, Doctor?

A. That is used for a neuropathic-type pain. Most commonly, diabetics that have -- their diabetes has progressed, and they have kind of numbness, tingling pain in their fingers; they'll use Gabapentin.

In this instance, he had -- he had some residual weakness after -- you know, again, he was found kind of in a contorted position. He complained of left arm weakness and some pain related to that, so they utilized Gabapentin for that upon discharge.

Q. That was in the prior hospitalization in April, April 2020; correct?
A. Correct.
Q. Okay. There was also Zolpidem; right?
A. Correct.
Q. Is that a sleeping pill?
A. That's Ambien, yes.
Q. Ambien?
A. Sleep aid.
Q. Can it be pretty powerful?
A. If you take enough of them, yes, it has detrimental effects; but otherwise, taken as prescribed, it's safe.
Q. Just like fentanyl, when taken in a prescribed dose, is not fatal; right?
A. When utilized under the care of a doctor, yes.
Q. Correct. When you use fentanyl not under the care of a doctor, it can be exceedingly lethal; right?
A. Correct.
Q. Thank you.

Then there was two others. I won't bother to go into them. So let me focus on -- if I may have a second.

Doctor, your opinion in this case, taking everything into account, was that this individual died from a lethal intake of fentanyl; correct?
A. Correct.

Q. And that nothing else contributed to that death; right?
A. Correct.
Q. And you eliminated the possibility of a stroke a month earlier because he didn't show vasculitis when you did the autopsy of the brain?
A. That's two separate issues there.
Q. Okay. Help us, please.
A. So the -- the initial radiology described, you know, the possibility of a stroke or an infarct in a particular region.
Q. Sorry to interrupt you again. I'm going to keep doing this.

The "initial radiology," please explain to us what that would mean.

A. So they -- he had multiple scans of his head, of his brain, using CT, computed tomography or MRI. The -- he had multiple MRI examinations over the course of several days when he presented.

And it was -- the portions of the study where they look at kind of how -- you know, kind of how the contrast material, you know, that they use in there, was -- would diffuse out through the vasculature. And there were some findings on those particular phases of the study, MRI gets really technical, but there were issues that they saw where

the -- there were some changes that suggested, hey, blood is not flowing properly or correctly or at all to this region. And -- and with vasculitis, again, similar kind of -- you know, kind of delays and diffusions is how they -- they describe it.

So based on those interpretations, they were -- they said they were suggestive of is there a possible infarct, is there a possible stroke, in particular in the right motor cortex causes his arm symptoms to the left side, and then he just kind of almost -- "multi-focal" is the phrase they use as far as the vasculitis picture there. So that was two different issues I'm looking for there. Vasculitis, I got to look at under the microscope.

If I'm looking for an infarct in the right motor cortex, I'm looking for, like I said, is the right side, is there something wrong versus the left side. Initially when you slice through it, you're looking for an infarct. One of the things you notice, even after you preserve it, you know, an infarcted area, it's soft. You know, it kind -- it gives. Even if the rest of the brain is nice and firm, that area stays soft. It's because necrotic tissue doesn't preserve as well.

I didn't see anything like that.

Q. Let me interrupt you. When you say "necrotic tissue," can you explain what that is.

A. Necrotic, basically dead, dying tissue.

Q. Okay. Thank you.

Please proceed.

A. Okay. So I didn't see any of that.

People who have had a stroke recover and it's been years since they had the ultimate end point of the process of the brain healing from a stroke, well, it actually forms a little cavity. Well, obviously, yeah, we're a month later, I wasn't going to see that.

So those are the big things, looking at initially. And then under the microscope, if I'm looking for areas of an infarct, I'm looking for various levels of inflammation not related to the vessels necessarily, but throughout the brain itself. There is kind of a -- we say spongiosis, but there is kind of like a -- it's not a nice solid sheet of brain, you know, the brain matter and all the cells that we see in there. There is holes in there now. I didn't see that either.

So I didn't see it grossly, didn't see it microscopically as far as an infarct. And then the vasculitis, I looked at multiple sections throughout the brain, not just, you know, the one, and surveying through all the vessels, I didn't see anything. And then when you talk about vasculitis, the inflammation tends to cuff around the vessels. I didn't see that.

Q. Let me ask you about that inflammation.
A. Uh-huh.
Q. Going back to April of 2020, with all those medical professionals attending to Mr. Reed a month earlier, they relied on radiology and a bunch of other tests; correct?
A. Yes. So radiology combined with clinical presentation.
Q. Precisely. And all these medical professionals looking at Mr. Reed -- and they kept him for a few days in the hospital for observation, and what have you -- concluded it had been a stroke; correct?
A. They -- that was part of their discharge diagnosis. Like I said, I looked for what they saw. I could not find it.
Q. Only what they diagnosed.
A. Yes.
Q. At discharge.

And to treat the -- a stroke, post-stroke symptoms, they prescribed steroids; right?
A. The steroids were primarily focused on the vasculitis, but in stroke cases, they will use it because the common thing there is swelling of the brain.
Q. Exactly.
A. So they want to help -- steroids help with that.

Q. And so they prescribed steroids to address the vasculitis, the swelling of the brain; correct?
A. Correct.
Q. And a month later -- or actually, more than a month later, when you actually started looking at the brain tissue, you saw no evidence of vasculitis; right?
A. Correct.
Q. Could that be due to the steroid effect?
A. I think that's most likely what happened, yeah. Versus, well, that was an incorrect diagnosis at the start.
Q. Did you have any reason to believe that the doctors in April 2020 missed, totally missed a diagnosis?
A. I have no reason to think that they missed the diagnosis of vasculitis. Without having spoken to those doctors, one of the questions I had, particularly with the left arm weakness, is he also had an initial diagnosis of rhabdomyolysis, and that's a breakdown in the muscle. He was found kind of in a contorted position with the armed pinned underneath.
Q. Doctor, when you say he was found, you're talking about one month later, May 21st?
A. No. I'm talking about in April when he was admitted.
Q. When he had the incident?
A. The incident.
Q. Go ahead.

A. So my question at the time for -- you know, related to that is, if he had rhabdomyolysis, did he also have an element of what we frequently see with rhabdomyolysis, repeated injuries in the military. Some call it compartment syndrome.

And what it is is that muscle breaks down. There is fluid that builds up. When you think about the structures of the arm and everything like that, and it builds up and puts pressure on things in there, puts pressure on the vessels, decreases blood flow, makes thing worse with that.

The other thing that doesn't like pressure is the nerves in your arm. So when you're talking about left arm weakness, what I would -- the question that I had for the clinicians that took care of him, well, did you rule out, okay, yeah, you focused on here. It's a whole series of dots, the peripheral nervous system, all the way down to the muscle that could have caused weakness. Is that why he had --

Q. Let me interrupt you.

When you say the questions you had for those clinicians from April 2020, did you call them and ask them?

A. Specific to that, no.

MR. CORTEZ: Thank you.

No more questions.

THE COURT: Okay. Redirect Examination?

MS. FORREST: Yes, Your Honor.

REDIRECT EXAMINATION

BY MS. FORREST:

Q. Dr. Platt, just to make sure we're all on the same page, is a stroke something different than vasculitis?

A. Yes. We're talking about stroke, cerebral vascular accident versus vasculitis, you know, is simply just inflammation associated with the vessels.

Q. And based on your examination of Mr. Reed's brain, you did not find any evidence of a stroke; is that right?

A. Correct.

Q. And based on your examination of Mr. Reed's brain, you determined that it is possible Mr. Reed had vasculitis, was treated for that with steroids and that that resolved; is that right?

A. Correct.

Q. And so your conclusion is that neither a stroke nor vasculitis in any way contributed to Mr. Reed's death; is that right?

A. That's correct.

Q. Defense counsel asked you questions about 6 through 10. These are the substances listed on the sixth page of your autopsy report.

A. Correct.

Q. And I believe you said that these are substances that

Mr. Reed was prescribed. They are prescription medications; is that right?

A. Yes.

Q. And did any of these four substances, so Substances 6 through 10, in your opinion, in any way contribute to Mr. Reed's death?

A. No.

MS. FORREST: No further questions for this witness, Your Honor.

May Dr. Platt be excused?

THE COURT: Okay.

MR. CORTEZ: No recross. And he may be excused. Thank you, Doctor.

THE COURT: Okay. Thank you, sir. You may step down.

THE WITNESS: Thank you.

MR. ROTH: Your Honor, before calling our next witness, the government asks permission to read into the record a stipulation as to the content of testimony that one witness would have offered. This has been agreed to by the parties.

THE COURT: Yes. Please, go ahead.

MR. CORTEZ: No objection.

MR. ROTH: Thank you, Your Honor.

If called to testify at trial, Austin Ritter would

testify that in light of his friendship with C.M.R., he knew that C.M.R. drove a dark-colored BMW sedan, and that the sedan had a dent in the area of the front bumper on the right-hand side.

Thank you.

Your Honor, the United States calls Stephen Logan.

THE COURT: Okay. While Mr. Logan is coming, ladies and gentlemen, I instruct you that the parties have agreed that Austin Ritter would so testify to the subject matter of the stipulation. You may consider that testimony in the same way as if it had been given here in court. So that's your consideration of the stipulated testimony.

THE CLERK: Please raise your right hand.

STEPHEN LOGAN, a Government witness, having been duly sworn, testified as follows:

THE WITNESS: Yes, ma'am.

THE CLERK: Thank you. If you can please state your name and spell it for the record.

THE WITNESS: My name is Stephen Logan. S-t-e-p-h-e-n L-o-g-a-n.

MR. ROTH: May I proceed, Your Honor?

THE COURT: Yes.

MR. ROTH: Thank you.

DIRECT EXAMINATION

BY MR. ROTH:

Q. Good afternoon, sir.

A. Good afternoon.

Q. Mr. Logan, what do you do for a living?

A. I'm a special agent with NCIS.

Q. Is that the Naval Criminal Investigative Service?

A. Yes, sir, it is.

Q. How long have you been an NC -- and what is it that you do for NCIS? I'm sorry.

A. Conduct criminal investigations as it relates to the Department of the Navy assets, property and personnel.

Q. And forgive me if you had said it, you're a special agent?

A. Yes, sir. I'm a special agent. I'm sorry.

Q. No. That's my mistake.

How long have you been a special agent with NCIS?

A. Fourteen years.

Q. Prior to joining NCIS as a special agent, what did you do?

A. I was in the Marine Corps Reserves and a professional firefighter in Kentucky.

Q. How long were you in the Marine Reserves?

A. Fourteen years.

Q. Did you ever deploy as a Marine?

A. Yes, sir, I did.
Q. How many times did you do that?
A. Three.
Q. Special Agent, what's your educational background?
A. I have a Bachelor of Science degree in arson investigation.
Q. Would you like some water?
A. No. I'm good. Thank you.
Q. Okay. Special Agent, what is the principal investigative mandate for NCIS?
A. Again, to conduct criminal and counterintelligence investigations as it relates to the Department of Navy assets, property and personnel.
Q. And your principal responsibility as a special agent is to carry out investigations in furtherance of the mandate; is that fair to say?
A. Yes, sir. That's correct.
Q. Okay. What kinds of matters have you investigated as an NCIS special agent?
A. I've investigated crimes against children, adult sex crimes, narcotics, organized crime and counterintelligence matters.
Q. As to narcotics, how many different kinds of investigations do you estimate you've done over the course of your career with NCIS?

A. Approximately 20.
Q. And what kinds of investigative techniques have you used in the course of those 20-odd investigations?
A. I've conducted both physical and electronic or technical surveillance. I've utilized search warrants, subpoenas. Debrief of defendants post-conviction. I've used confidential informants and undercover agents in furtherance of those investigations.
Q. Do you also use subpoenas to obtain information?
A. Yes, sir, we do.
Q. And do you confer with other experienced law enforcement officers who work in drug trafficking?
A. Yes, sir.
Q. Okay. Special Agent, as part of your work, have you ever conducted a search of a phone?
A. Yes, sir, I have.
Q. How many times do you think you've done that?
A. Probably a hundred.
Q. Okay. And have you ever conducted a search of a social media account?
A. Yes, sir, I have.
Q. How many times do you think you've done that?
A. Probably 25 or so.
Q. Okay. Did you receive any formal training as part of joining NCIS?

A. Yes, sir, I did.
Q. And have you received formal training throughout the course of your 14 years?
A. Yes, sir, I have.
Q. Would you describe, in a summary way, the training you've received over the past 14 years.
A. So initially I was trained at the Federal Law Enforcement Training Center in Brunswick, Georgia. It's a criminal investigations training program and then an NCIS-specific-based special agent basic training program. I've received additional specialized training in death investigations, crime scene processing, proactive criminal operations, like narcotics, things of that nature.
Q. And is it fair to say that on-the-job training happens at the agency?
A. That is the primary form of training, yes, sir.
Q. Special Agent, as part of your training and your experience, have you become familiar with a law known as the Controlled Substances Act?
A. Yes, sir, I have.
Q. Is it fair to say that's a law that controls certain substances?
A. Yes.
Q. From your perspective as an investigator, how does the Controlled Substances Act go about controlling substances?

A. So the Controlled Substances Act evaluates each drug and determines its value, or I guess -- lack of a better term, its value on the society, and places it on the schedule.
Q. So when you say "value on society," is it fair to say that's the, you know, salutary effects and also the potential for abuse?
A. Yes, sir.
Q. Okay. And so this Act, in evaluating those substances on those criteria, I think you said it schedules them; right?
A. Yes, sir, it schedules them.
Q. How many schedules are there under the Act?
A. There are five.
Q. Okay. And so are they numbered?
A. Yes, sir. I through V.
Q. How do substances that are listed on Schedule I differ from substances on Schedule V?
A. Substances on Schedule I have no medical use and have a high propensity for abuse.
Q. And fair to say that Schedule V is diametrically the opposite?
A. Correct.
Q. So it's sort of a sliding scale across the five schedules; right?
A. Right. That is correct.

Q. Okay. Just for the record, let me ask and then you answer. Okay?
A. Yes, sir.
Q. Okay. Great.
Special Agent, have you come across a drug called oxycodone in the course of your career?
A. Yes, sir, I have.
Q. What kind of a drug is oxycodone?
A. It is an opiate.
Q. Okay. And is it scheduled?
A. Yes, sir, it is.
Q. What schedule is oxycodone on?
A. It is a Schedule II.
Q. And so as a Schedule II, does that mean it can be distributed, but it's got to be by a medical professional?
A. Yes, sir, that's what that means.
Q. All right. Have you heard of a drug called fentanyl?
A. Yes, sir, I have.
Q. What kind of a drug is fentanyl?
A. Fentanyl is also an opiate.
Q. Okay. Is it scheduled?
A. Yes, sir.
Q. And what schedule is fentanyl on?
A. Fentanyl is also a Schedule II drug.
Q. How does fentanyl compare to oxycodone, as you

understand it?
A. Fentanyl is a more powerful pain reliever than oxycodone.
Q. Is it fair to say that it is dramatically more powerful?
A. Yes, sir.
Q. Okay. Special Agent, have you heard of M30 pills?
A. Yes, sir, I have.
Q. What are M30 pills in their genuine form?
A. M30 pills in their genuine form are oxycodone, 30-milligram oxycodone tablets.
Q. What do they look like?
A. They are round and blue.
Q. Do they have any markings on them?
A. They do.
Q. What markings are on M30 pills?
A. They have an M imprinted on one side and the number 30 imprinted on the other.
Q. Now, you said just now in their genuine form, it's 30 milligrams of oxycodone in an M30 pill; is that correct?
A. Yes, sir.
Q. Is it your understanding that in a genuine form, every single pill that comes off the line at the pharmaceutical plant is going to have 30 milligrams of oxycodone in it?
A. That's correct.

Q. In the course of your work as an investigator, have you come across another form of an M30 pill?
A. Yes, sir, I have.
Q. Would you please describe that.
A. The other form that I've encountered is a counterfeit M30 pill.
Q. And in your experience, how do the counterfeit M30 pills differ from the genuine ones?
A. The opiate in the counterfeit M30 pills is fentanyl.
Q. Okay. Do those M30 pills contain substances known to you as bonding agents, or cutting agents?
A. Yes, sir.
Q. And what is, if you will, the chemistry going on with the bonding agent and the fentanyl in a counterfeit M30 pill?
A. Probably the -- I'm by no means a chemist, but probably the easiest way to describe it is if you're making meatloaf, you put bread crumbs or cracker crumbs in there, in with the eggs to hold the meatloaf together; and the cutting agents or bonding agents work very similar.
Q. Special Agent, have you heard of Percocet in the course of your career?
A. Yes, I have.
Q. What is Percocet?
A. Percocet is a brand name for oxycodone.

Q. Okay. So all Percocet are oxycodone, but not all oxycodone is Percocet; is that right?
A. That is correct.
Q. Just to be clear, Percocet is a Schedule II controlled substance; right?
A. That is correct.
Q. Oxycodone is a Schedule II controlled substance; right?
A. That is correct.
Q. Fentanyl is a Schedule II controlled substance; right?
A. That is correct.
Q. Thank you.

Special Agent, in the course of your 14 years, and based on your training and experience, have you become familiar with common terms used in drug trafficking?
A. Yes, sir, I have.
Q. I'd like to ask you a couple of questions about that based on your experience, and then I'm going to stop that part of your testimony and ask you to pivot, focusing on the facts of this case. Okay?
A. Yes, sir.
Q. In general, have you heard the term "trap" before?
A. I have.
Q. What does "trap" mean?
A. "Trap" is usually associated -- usually in conjunction

with the word "trap house" or something to that effect, and a trap house is a place where drugs are sold.

Q. Okay. Have you heard the term "plug" before?

A. Yes, sir, I have.

Q. What is a plug?

A. A plug is a source of supply for drugs or a drug dealer.

Q. So somebody who plugs you?

A. Yes.

Q. Okay.

A. Yes, sir.

Q. Thank you.

Special Agent, have you heard of any shorthand for Percocet?

A. Yes, sir, I have.

Q. What are common shorthands for Percocet that you've come across?

A. Percs, 30s, blues. Those are the three that come to mind right off the top of my head.

Q. Okay. Thank you. And Special Agent, I do just want to ask one thing, and I should have followed up on this point.

In your experience, have you seen the results of drug testing of counterfeit M30 pills, the one that has the fentanyl in them?

A. Yes, sir, I have.

Q. In your experience, are they always consistent, with respect to the amount of fentanyl in the pill?
A. No, sir, they are not.
Q. Okay. So it's not like when it gets manufactured at the pharmaceutical company; right?
A. Correct.
Q. Okay. Special Agent, I'm going to now ask you to answer a series of questions not with respect to your training and experience, but rather just focusing on this case. Okay?
A. Okay.
Q. Thank you.

Special Agent, you are aware that the United States has brought a case against a person by the name of Nameer M. Atta; is that correct?
A. Yes, sir, I am.
Q. Do you know what Mr. Atta looks like?
A. Yes, sir, I do.
Q. How do you know that?
A. Through surveillance, looked at his driver's license, and also I had two occasions to speak to Mr. Atta.
Q. So you've personally visited with Mr. Atta face-to-face?
A. Yes, sir.
Q. Do you know what Mr. Atta looks like?
A. Yes, sir, I do.

Q. And do you see him in the courtroom today?
A. Yes, I do.
MR. CORTEZ: Stipulate to identity.
THE COURT: Do you accept the stipulation?
MR. ROTH: I'd like to do it, Your Honor.
THE COURT: Okay. Go forward.
Q. BY MR. ROTH: Special Agent, would you please point out Mr. Atta and identify him by an article of clothing he's wearing?
A. Mr. Atta is sitting to your left at the table wearing the light-colored shirt.
MR. ROTH: And, Your Honor, may the record please reflect that Special Agent Logan has identified the defendant as Mr. Atta.
THE COURT: Yes. The record will so reflect.
MR. ROTH: Thank you.
Q. Special Agent, have you conducted an investigation into the death of a man named Christian Reed?
A. Yes, sir, I have.
Q. Who was Christian?
A. Christian was an active-duty Marine, assigned to Marine Air Corps Station in Miramar here in San Diego County.
Q. Where did Christian die?
A. Christian died in his barracks room.
Q. When did he die?

A. On 21 May -- May 21st, 2020.
Q. How old was he?
A. He was 26 and a day.
Q. So he died the day after his 26th birthday?
A. Yes, sir.
Q. Thank you.
Special Agent, why did NCIS commence an investigation into Christian's death?
A. It is NCIS policy to conduct an investigation related to all noncombat deaths of active-duty service members, both in the United States and abroad.
Q. And as the investigation began, what initial steps did NCIS take?
A. NCIS agents conducted what we call a scene examination. They respond to the location where the individual died and memorialize that space or the area with both photographs -- with photographs, sketches, notes and also look for and seize any evidence.
Q. In that initial search, documentation process of Christian's room, did the agency find any particular item of evidence that shed light on how Christian came to die?
A. Yes, sir, they did.
Q. What did you find?
A. Christian's cell phone.
Q. And what was the day of that initial search?

A. That day was on the day he died, May 21st, 2020.
Q. Okay. Now, at some point, is it fair to say that NCIS was notified that the medical examiner had determined the cause of death was a fentanyl overdose?
A. Yes, sir.
Q. When did NCIS receive that notification?
A. That was in August of 2020.
Q. And did the agency take any follow-on steps upon receiving that notification?
A. Yes, sir, we did.
Q. What did you do?
A. Myself and another agent went and conducted a secondary search of Christian's barracks room.
Q. And why did you do that?
A. We were looking for any fentanyl or anything like that that would have given us any indication or identified a location of fentanyl.
Q. Had the room been cleaned by the time you went to it in August of 2020?
A. Yes, sir, it had.
Q. And did you find any fentanyl or fentanyl pills in the room at that point?
A. No, sir, we did not.
Q. Okay. Now, Special Agent, at the time of Christian's death, there were prescription bottles in his room; right?

A. Yes, sir, that's correct.
Q. Okay. A variety of prescriptions; fair enough? Fair to say?
A. Yes, sir.
Q. Were those pill bottles seized at the time of the initial search on May 21st of 2020?
A. No, sir, they were not.
Q. And why not?
A. Because the pills in the bottle matched what was on the label.
Q. So in other words, there was no evidence at the time of death of any illicit drug use that the agency saw from examining the pill bottles and their contents?
A. Yes, sir, that's correct.
Q. Okay. Special Agent, you should have a fairly large binder in front of you. Do you see that binder?
A. Yes, sir, I do.
Q. Okay. If you would, could you please turn to the tab that is No. 88.
A. No. 8?
Q. 88. Double 8s.
A. Double 8s.
Okay. I'm there.
Q. Okay. And have you reviewed the exhibit marked as Government's 88?

A. Yes, sir, I have.
Q. Do you recognize this image?
A. Yes, sir, I do.
Q. What is depicted in Government Exhibit 88?
A. It's a photograph of a shelf in Christian Reed's barracks room.
Q. And how do you know that this is a photograph of a shelf in Christian Reed's barracks room?
A. Because it was included in our case file, and I've reviewed the case file several times.
Q. Is this, Government's Exhibit 88, a fair and accurate copy of that photograph?
A. Yes, sir, it is.

MR. ROTH: Your Honor, the United States moves to admit and publish Exhibit 88.

MR. CORTEZ: No objection.

THE COURT: 88, it's already received in evidence.

MR. ROTH: I'm sorry to everyone. If we could bring up Government's Exhibit 88.

Q. Okay. Special Agent, do you see a phone in the center of the having already been admitted Government's 88?
A. Yes, sir, I do.
Q. Do you recognize that phone?
A. Yes, sir, I do.
Q. Okay. Whose phone is that?

A. That is Christian Reed's cell phone.
Q. Okay. Did you take physical possession of his phone?
A. Yes, sir, we did.
Q. Okay. And did you provide this phone to any other member of the law enforcement community for any reason?
A. Yes, sir, I did.
Q. What did you do?
A. I provided this phone to one of our digital forensic examiners at NCIS.
Q. And what did the digital forensic examiner do with Christian's phone?
A. He made a digital copy of Christian's phone.
Q. Okay. And did you receive the digital copy?
A. Yes, sir, I did.
Q. Special Agent, can you just provide, from your perspective, an explanation of what the digital copy is.
A. Okay. Sure. So the digital copy, very simply, is they plug the phone into a computer. They use specific digital tools and make a copy of the cell phone. It's an exact replica.
Q. Special Agent, if you would please now turn to Exhibits 9, 10, 11 and 12. Please look up when you're ready.
A. Yes, sir.
Q. Okay. Do you recognize Government's Exhibits 9, 10, 11 and 12?

A. Yes, sir, I do.
Q. What are these?
A. Exhibit 9 is a photograph of Christian Reed in his dress-blues uniform. The second photo in Exhibit 9 is a photo of Christian Reed in his Marine Corps combat utility uniform.

Exhibit 10 is a photograph of Christian in his -- in a gray T-shirt with an orange "T" on it.

And you asked for 8, 9 and 10; is that correct?
Q. And 11 and 12.
A. Sorry. I skipped No. 8. No. 8 --
Q. No, we're not doing 8. 9, 10, 11 and 12.
A. Oh, I'm sorry. 11 and 12.

11 and 12 are photographs of Christian's car.
Q. Okay. How do you know that 9, 10, 11 and 12 come from Christian's phone?
A. Because I reviewed the forensic copy of Christian's phone, and I observed these photos.
Q. Are 9, 10, 11 and 12 fair and accurate copies of the images that you saw in your review of the download of Christian's phone?
A. Yes, sir.

MR. ROTH: Your Honor, we would move to admit and to publish 9, 10, 11 and 12.

THE COURT: Okay. To be clear, Exhibit 9 is two

pages; correct?

MR. ROTH: It is, Your Honor.

THE COURT: Okay. Any objection to 9, 10, 11 and 12?

MR. CORTEZ: No.

THE COURT: They are received in evidence.

(Government's Exhibits 9, 10, 11 and 12 received in evidence.)

MR. ROTH: Thank you. If we can please bring up Government's Exhibit 9.

Q. This is Page 1 of Government's Exhibit 9. Special Agent, who is in this picture?

A. That's Christian Reed.

Q. How do you know that?

A. So several reasons. According to initial death examination, agents on scene were able to identify Christian against his DOD ID card. And then autopsy, they identified Christian via DNA, fingerprints and dental records.

Q. And have you had a chance to review the photos of the body that the ME examined by DNA, dental records, fingerprints?

A. Yes, sir, I have.

Q. And was it Christian?

A. Yes, sir, it was.

Q. If we can scroll to the second page.

Is this Christian again?

A. Yes, sir, it is.

Q. How does the uniform that he's wearing differ -- or the clothes that he's wearing differ from the earlier page?

A. This is what we call a combat utility uniform.

Q. Thank you. If we can please bring up Exhibit 10.

Special Agent, is this Christian?

A. Yes, sir, it is.

Q. Could you describe the T-shirt that he's wearing?

A. It's a gray T-shirt with an orange "T" on it.

Q. Do you recognize that orange T?

A. Yes, sir. It is the emblem or symbol for the University of Tennessee.

Q. Okay. So that's a Tennessee Volunteers T-shirt; right?

A. Yes, sir.

Q. Special Agent, from your review of the phone download, are you aware of the date stamp that's associated with this photograph?

A. Yes, sir, I am.

Q. On what date, per the download, was this photo taken?

A. That was May 21st, 2020.

Q. So that's the day that Christian died; right?

A. Yes, sir, that's correct.

Q. Okay. We'll come back to that point in a bit. If we

can turn to Government's Exhibit 11 and bring it up, please.

Special Agent, this is a dark blue BMW; right?

A. Yes, sir, that's correct.

Q. Do you know who drove this car?

A. Yes, sir, I do.

Q. Who was the driver of this car that's depicted in Government's Exhibit 11?

A. Christian Reed.

Q. Okay. And how do you know that?

A. It was identified by his friend Austin Ritter, and I also pulled this photo from Christian Reed's car -- phone.

Q. I'm sorry?

A. I'm sorry. I pulled it from his phone. Not his car, I apologize.

Q. No. It's fine.

Special Agent, do you see a building in the background?

A. Yes, sir, I do.

Q. What kind of a building is that?

A. That's a barracks building on Marine Corps Air Station Miramar.

Q. Okay. If we can pull up Government Exhibit 12.

And Special Agent, how does Government's Exhibit 12 differ from Government's Exhibit 11?

A. 11 was more -- was a photo of the driver's side. This

is a photo of the passenger side.
Q. Okay. Anything notable or distinctive about this BMW as depicted in Government's Exhibit 12?
A. Yes, sir, there is.
Q. What is it?
A. There is damage to the bumper.
Q. Okay. Fair to say that below the headlight, there is a depression on sort of the right-edge side of the bumper?
A. Yes, sir, that's correct.
Q. Okay. Special Agent, do you happen to recall the approximate date of this, Government's Exhibit 12?
A. It was May 19th, 2020.
Q. Okay. So that's two days before the death?
A. Yes, sir.
Q. Okay. Special Agent, if you would now please go to the binder and look at Government's Exhibit 13. And just look up when you're ready.
A. Yes, sir.
Q. Okay. Special Agent, what is Government's Exhibit 13?
A. Exhibit 13 is a GPS satellite photo of 9845 Jake Lane.
Q. Do you recognize this image at Government's Exhibit 14 -- 13, I'm sorry. Do you recognize Government's Exhibit 13?
A. Yes, sir, I do.
Q. How so?

A. I reviewed Christian's cell phone, and that image was pulled from Christian's cell phone.

Q. Is this a fair and accurate copy of the satellite map image that you found in Christian's -- in the download of Christian's phone?

A. Yes, sir.

MR. ROTH: Your Honor, move to admit Government's Exhibit 13.

THE COURT: Any objection?

MR. CORTEZ: No objection.

THE COURT: 13 is received.

(Government's Exhibit 13 received in evidence.)

MR. ROTH: Thank you, Judge. If we can bring up 13, please.

Q. So, Special Agent, we're now looking at, as you described, a sort of satellite image; right? Do you see in the center of this image a large blue circle?

A. Yes, sir, I do.

Q. Okay. And then within that blue circle, there is a white circle that is sort of almost more hand drawn?

A. Yes, sir.

Q. And above that is a white arrow, correct?

A. Correct.

Q. Okay. Do you see a pin drop just at the bottom edge of the white circle?

A. Yes, sir, I do.
Q. All right. Special Agent, what is the address associated with this pin drop?
A. 9845 Jake Lane.
Q. How do you know that?
A. Because I have conducted surveillance at that location and also served a search warrant at that location.
Q. And Special Agent, we will come to this more fully in your testimony, but who lives at 9845 Jake Lane?
A. Nameer Atta.
Q. All right. Now, Special Agent, you learned upon receipt of the opinion of the medical examiner that there was an opinion this was a fentanyl overdose; correct?
A. Yes, sir.
Q. What investigative steps, if any, did you take with respect to Christian's phone with that information in mind?
A. I reviewed text messages and other information in that phone to identify a source of supply for fentanyl.
Q. And did you find communications on Christian's phone tending to suggest that he had been engaged in transactions that led to his death?
A. Yes, sir, I did.
Q. Over what kind of medium, if you will, did those communications occur?
A. Via Snapchat.

Q. All right. Special Agent, I'm going to have you jump to Government's Exhibit 15.
A. 1-5?
Q. Please.
Do you recognize Government's Exhibit 15?
A. Yes, sir, I do.
Q. And for the record, Government's Exhibit 15 is several pages. What is Government's Exhibit 15?
A. Those are photographs I took of Snapchat messages on Christian Reed's cell phone.
Q. And can you explain -- how do I put this? Are these fair and accurate depictions of the communications you saw on Christian's phone?
A. Yes, sir, they are.
MR. ROTH: Okay. Your Honor, I would move to admit Government's Exhibit 15.
MR. CORTEZ: Subject to our earlier objections, nothing else.
THE COURT: Okay. 15 is a multipage exhibit. Do you have a total page number?
MR. ROTH: Yes, of course, Your Honor. Eight pages, Your Honor.
THE COURT: Okay. Government's 15, which is comprised of eight pages, is admitted.
(Government's Exhibit 15 received in evidence.)

MR. ROTH: If we could please bring up Government's Exhibit 15. If we could please zoom in on the first photo there. Perhaps to put just the whole thing on the screen. Perfect. This is perfect.

Q. Okay. Special Agent, a couple of things. First of all, this is Christian's phone; correct?

A. Yes, sir.

MR. CORTEZ: Asked and answered, Your Honor.

THE COURT: Overruled. The answer will stand.

MR. ROTH: I'm sorry, Your Honor. I'm sorry.

Q. Whose thumb is this?

A. Whose thumb is this? That's mine.

Q. Okay. Do you see at the top, there is a little icon of a head with some hair and some texts next to it?

A. Yes, sir.

Q. What is written next to that little icon at the top of the screen?

A. Next to the avatar is the words "My G" and it has an icon for an electrical plug.

Q. Okay. So "My G" with, fair to say, that's an emoji?

A. Yes, sir.

Q. And the emoji is of a plug; correct?

A. Yes, sir.

Q. I'd like to go through the first message. Could you please read the first message that's indicated here.

A. The first message reads: Send @ of who referred to verify yourself. And send school/area for delivery reference. Do not leave on read. Thanks! DLG, handshake emoji.

Q. Okay. Special Agent, what do you understand the phrase "verify yourself" to mean here?

A. The individual that sent that message is asking for bona fides.

Q. Okay. Now, in response, Christian gives -- he gives a response; right?

A. Yes, sir, he does.

Q. And what's the response?

A. Christian responds: @tg_cloter and Miramar/San Diego area.

Q. Special Agent, did you conduct any follow-up investigation into this @tg_cloter, which appears to be provided in response to the request for verification?

A. Yes, sir, I did.

Q. Okay. What did you learn?

A. I learned that tg_cloter is an individual known as Carson Cloter.

Q. Did you conduct any investigation to assess whether Carson Cloter might have provided fentanyl or oxycodone to Christian Reed?

A. Yes, sir, I did.

Q. What did you learn in the course of your investigation?

MR. CORTEZ: Objection. Hearsay.

THE COURT: Sustained. Lack of foundation.

MR. ROTH: Okay. Fair enough.

Q. Did you conduct an investigation?

A. Yes, sir, I did.

Q. Did you conduct interviews?

A. I did.

Q. And did you gather information from your interviews and from your review of information in general?

A. Yes, sir, I did.

Q. And do you have an understanding of what you believe Mr. Cloter's role is here?

A. Yes, sir, I do.

Q. Do you believe Mr. Cloter provided any fentanyl or oxycodone?

MR. CORTEZ: Objection. Speculative and hearsay.

THE COURT: Sustained.

MR. ROTH: Okay. We'll move on.

Q. Special Agent, "My G plug," what do you understand that to be?

A. That is how Christian saved this person's name in his phone, or in his Snapchat.

Q. Okay. I'd like to just scroll through the eight pages

and have a look at them. And Special Agent, if we could go to the next page.

Special Agent, can you summarize the nature of the communications between My G plug and Christian.

So let's stop for a second. Do you see about three quarters of the way down, there is a check mark, and below it some writing?

A. Yes, sir, I do.

Q. What is that writing?

A. Say that again? I'm sorry.

Q. What does it say?

A. It says $70 - 2 Perc30's/PU at 7/11 PQ.

Q. What is your understanding of that message?

MR. CORTEZ: Objection. Hearsay. Lacks foundation.

THE COURT: Overruled.

Q. BY MR. ROTH: What is your understanding of that message?

A. That message is a drug transaction for two 30-milligram Percocets for $70 to be picked up at the 7-Eleven on Penasquitos Drive.

Q. And specifically, 30-milligram Percocets?

A. Yes, sir.

Q. Okay. Great. If you could scroll to the next page.

Fair to say that it's similar?

A. Yes, sir.
Q. Keep going.
Continuously the same?
A. Yes, sir.
Q. If we can go to the last page of this one.
Special Agent, do you see the last entry on this set of communications?
A. Yes, sir, I do.
Q. Could you just -- what does it say there at the bottom?
A. It says, !! $300- 12Perc30's/PU Casa.
Q. Okay. Special Agent -- we can take 15 down. Thank you.
Did you find these communications on the download of Christian's phone?
A. Yes, sir, I did.
Q. In other words, you found them two ways; right?
A. Yes, sir.
Q. If you could please have a look at Government Exhibit 14. And look up when you're ready.
A. Yes, sir.
Q. Do you recognize Government Exhibit 14?
A. Yes, sir, I do.
Q. What is it?
A. It is part of the forensic download of Christian Reed's cell phone.

Q. And what specifically is it from the forensic download from Christian Reed's cell phone?
A. It is the Snapchat messages between Mr. -- Christian Reed and My G.
Q. Okay. Are these a fair and accurate depiction of the communications between Christian and My G plug found on the download of Christian's phone?
A. Yes, sir, they are.
MR. ROTH: Your Honor, we would move to admit Government's Exhibit 14.
MR. CORTEZ: Subject to earlier objections. That's all I have.
THE COURT: Okay. 14 is received.
(Government's Exhibit 14 received in evidence.)
MR. ROTH: And, Your Honor, this one is one, two, three, four, five pages, Your Honor.
THE COURT: Okay. Thank you.
MR. ROTH: Thank you. If we could bring up Government's Exhibit 14.
Q. Special Agent, the first -- in row one, do you see the column that says "body"?
A. Yes, sir, I do.
Q. Okay. Is the message there the message that you read, the first message from Exhibit 15 that you just described?
A. Yes, sir, it is.

Q. Okay. And is the response in row two, that's the same response?
A. Yes, sir, that's correct.
Q. Okay. Special Agent, have you compared 14 and 15?
A. Yes, sir, I have.
Q. In terms of content, are they the same thing?
A. Yes, sir, they are.
Q. But there is additional information in 14; right?
A. Yes, sir, that's correct.
Q. And I'd like to talk about that. First of all, do you seem columns labeled "from" and "to"?
A. Yes, sir, I do.
Q. And if we could start with row one. Do you see the My G plug text in that -- in the first row?
A. Yes, sir, I do.
Q. Is there another name associated with My G plug there?
A. Yes, sir, there is.
Q. What's written there?
A. It is nameeratta3324.
Q. What is your understanding of the association of the phrase nameeratta3324 with My G plug?
A. Nameeratta3324 and My G plug are the same person.
Q. Okay. So My G plug is how Christian saved the account, nameeratta3324, in his Snapchat; is that right?
A. Yes, sir, that's correct.

Q. Okay. If you look in the next column, do you see "participants"?
A. Yes, sir, I do.
Q. And do you see, in addition to the nameeratta3324, there is a second account listed; right?
A. Yes, sir, there is.
Q. And what's that other account?
A. Chreed06 with parentheses, owner.
Q. Okay. We'll come to this more fully, but, Special Agent, did you come to establish who chreed06 was?
A. Yes, sir, I did.
Q. Who was that?
A. Christian Reed.
Q. And did you come to identify evidence showing who nameeratta3324 is?
A. Yes, sir, I did.
Q. And who is that?
A. Nameer Atta.
Q. Okay. The defendant?
A. Yes, sir, the defendant.
Q. Special Agent, what is the date of the first message exchanged between the defendant and Christian Reed here?
A. April 14th, 2020.
Q. And if we can please go down to the last page and the final entry.

Special Agent, what's the date of the last entry or the last exchanges between Christian and the defendant?

A. That date is May 20th, 2020.

Q. And could you remind me, when did Christian die?

A. May 21st, 2020.

Q. Thank you. Special Agent -- we can take 15 down. Actually, if I can go back to Page 1, please.

In row seven, do you see an incoming message from nameeratta3324 to chreed06?

A. Yes, sir, I do.

Q. And what's written in the incoming message?

A. It's an address.

Q. What's the address?

A. 9835 Jake Lane, San Diego, California 92126.

Q. Have we seen this Jake Lane address already in your testimony today?

A. We saw 9845 Jake Lane.

Q. Okay. How did 9845 Jake Lane relate to 9835 Jake Lane?

A. Those buildings are adjacent to one another.

Q. Thank you. We can take down Government's 14.

Special Agent --

THE COURT: Mr. Roth, are you going move to a separate line of inquiry at this point?

MR. ROTH: I am. Actually, if I may -- I'm so

sorry. If I could put in one more exhibit, and then we can stop.

THE COURT: Sure.

MR. ROTH: Thank you.

Q. Special Agent, could you have a look at Government's Exhibit 16.

A. 16?

Q. Uh-huh. Just look up when you're ready.

Do you recognize Government's Exhibit 16?

A. Yes, sir, I do.

Q. What is it?

A. It is a banner that I observed in Christian Reed's cell phone.

Q. By "banner," do you mean Snapchat banner?

A. Yes, sir, it is a Snapchat banner.

Q. And is Government's Exhibit 16 a fair and accurate depiction of the Snapchat banner that you saw in the download of Christian's phone?

A. Yes, sir, it is.

MR. ROTH: Your Honor, we would move to admit Government's Exhibit 16.

MR. CORTEZ: Subject to earlier objections.

THE COURT: Okay. 16 is received.

(Government's Exhibit 16 received in evidence.)

MR. ROTH: And I would ask to just publish it

briefly. If we could bring it up.

Q. Special Agent, I just want to ask you to summarize the content of Government's Exhibit 16, and I think with that, we'll stop for the day.

So could you just please take us through what this is.

A. Sure. It's -- at the top of it, it says, Temple is Open. Tap in with orders. Taking off five to ten dollars off all minimum orders, electrical plug icon. Delivery and pickup available.

Q. Okay. And Special Agent, that's the same icon that we've seen when we were talking about My G plug when looking at Christian's Snapchat exchanges with the defendant; right?

A. Yes, sir. That's correct.

MR. ROTH: Your Honor, I can stop there.

THE COURT: Okay. Ladies and gentlemen, we're going to take our evening recess at this time. And before you leave, I remind you of the admonition not to form or express an opinion about this case, not to discuss it with anyone until it is submitted to you for deliberations with your fellow jurors.

And I also repeat the admonishment not to do any independent research about any facts and circumstances having to do with this case, the participants in this case, the

locations in this case or anything at all having to do with the testimony that you've heard so far in this matter.

So with that, ladies and gentlemen, I wish you all a very good evening. We will reconvene promptly at 9:00 a.m. tomorrow morning, and we will follow the schedule I previously referenced. So thank you very much for your attention, and we'll see you tomorrow morning. And you can leave your notebooks on your chair. Again, the area will be secure overnight.

One more thing, ladies and gentlemen. If you have car trouble, if you have child-care issues, if anything unexpected comes up, please contact Ms. Ortiz. I believe you have her contact information. If you don't, she will provide it to you. She will be your point of contact. So please telephone her in the morning if something comes up. I don't expect that it will, but if an emergency arises, she will be your point of contact.

So good evening.

(Jury absent, 4:54 p.m.)

THE COURT: We are outside the presence of the jury. All counsel are present. Mr. Atta is present.

Is there anything that we need to discuss before we break for the evening?

MR. CORTEZ: Not from us.

THE COURT: Okay.

MR. ROTH: Just a heads up, Your Honor. After Special Agent Logan, we have two more witnesses. They will be brief. I think that at the rate we're going, the government's case may end by the lunch hour tomorrow. And I have no idea what the defense plan is, but just for knowing.

And then, Your Honor, I do anticipate that screenshot that we've raised a couple of times today is going to come up tomorrow morning, so just for the Court's --

THE COURT: Okay. That's 43 and 44; correct?

MR. ROTH: That is correct.

THE COURT: Okay. Why don't we reconvene tomorrow morning at 8:45. We'll discuss those exhibits, and if anything comes up during your preparation tonight, we can discuss it before we bring the jury in at 9:00 a.m.

MR. ROTH: Okay.

MR. CORTEZ: Thank you.

THE COURT: Mr. Cortez, it's coming to that time in the trial --

MR. CORTEZ: It is.

THE COURT: -- where I'd like to know if you have a plan or how you see the defense case going. Do you intend to call witnesses, and how long would your presentation be if you call witnesses?

MR. CORTEZ: We will make that determination tonight and tomorrow morning. If we do, it will be an hour and a

half.

THE COURT: Okay. So it looks like we're going to close tomorrow.

MR. ROTH: Yes.

MR. CORTEZ: Oh, yeah. And counsel for the government had notified me last week that we should be ready to go Tuesday. He was kind enough to do that.

THE COURT: Okay. So what I'm going to do now is go back, finalize my preliminary draft jury instructions. The parties are relatively on the same page when it comes to the jury instructions, so there is not going to be a lot to discuss. But I will e-mail those to you within the hour.

And what I'll ask the parties to do is come back tomorrow at 8:30, promptly at 8:30, and we'll have our jury conference then so that the parties know what the jury instructions are going to be and you can formulate your closing arguments accordingly. And I'll have final packets with the jury instructions for you, if you want to use the Elmo, or I can give it to you in electronic format if, over the lunch period, you'd like to incorporate those into a PowerPoint or whatever presentation you intend to use for closing.

So let's reconvene tomorrow morning at 8:30, promptly at 8:30, because I don't want to hold the jury. At 9 o'clock, they are coming in whether we are done or not.

MR. CORTEZ: 8:30 it is. Thank you.

THE COURT: Thank you.

MR. ROTH: Thank you, Judge.

MR. CORTEZ: Have a good evening.

THE COURT: Thank you. You, too.

(Proceedings adjourned at 4:57 p.m.)

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C E R T I F I C A T I O N

I hereby certify that I am a duly appointed, qualified and acting official court reporter for the United States District Court; that the foregoing is a true and correct transcript of the proceedings had in the aforementioned cause; that said transcript is a true and correct transcription of my stenographic notes; and that the format used herein complies with the rules and requirements of the United States Judicial Conference.

DATED: October 25, 2022, at San Diego, California.

S/CAMERON P. KIRCHER
CAMERON P. KIRCHER